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HSM Notes

Health System Management (Kenya Medical Training College)

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HEALTH SYSTEMS MANAGEMENT

UNIT ONE: INTRODUCTION TO LEADERSHIP AND MANAGEMENT

Concepts of leadership and management

Leadership and management are often regarded as interchangeable which is not the case. While the most effective managers are usually also good leaders, this is not necessarily the case. Nor is a good leader always a good manager.

Management

Originally, the term management comes from Latin word "manus" referring to the driver controlling a team of horses pulling a cart. This image helps us in considering other definitions. The examples of definitions of management as defined differently by different scholars are that, it is the;

- Process of getting things done through other people (in simple terms).
- Process through which resources in an organization are made available and effectively used in accomplishing the organizational goals.
- Process of working with and through individual and groups and other resources to accomplish organizational goals or objectives.
- Planning, organizing, co-ordinating, commanding and controlling.
- Process of creating and maintaining of an internal environment of an organization in which individuals and groups perform effectively towards the attainment of the organization goals.
- Process of integrating (bring together) all the organizational aspects for instance the structure, people and technology in order to achieve the organizational goals.

It should be noted that, the definitions of management are non-exhaustive, they vary greatly because of the nature, and types of organizations in terms of products produced or services rendered. However, the most important elements in most definitions are: -

- Process
- Resources
- Utilization and
- Goals

Management is important because it enhances the efficiency and effectiveness of an organization's performance. It involves planning, leading, organizing and controlling an organization, as well as more the intangible qualities of inspiration and motivation. Regarding management as a process helps to ensure that operations achieve their objectives and that essential health systems are improved – the operational aspect of management.

Who Is a Manager and What Are Their Roles?

A manager is a person who has the responsibility for achieving certain outcomes, along with the authority to utilize the resources of the organization in doing so. The combination of responsibility and authority is crucial to the concept of management. Timeliness is another critical aspect. The timely use of the resources – whether human, financial, information or physical assets – is essential for effective management. In an ideal management team, the team members recognize the authority of the manager and provide support in a constructive way.

A manager is therefore a person who can organize people to work together harmoniously and make effective use of resources to achieve laid-down objectives, through a process that

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includes planning, implementation, monitoring and evaluation. A distinction should be made between a manager who is a leader, and one who is an administrator. Administration is a subset of management. An administrator is somebody who interprets policies and directives from above for implementation, knows the rules and applies them well. The administrator and the manager do not have to be different people.

Process Management

Process management means documenting processes (the way work should be done in the organization), measuring the results of that work as seen by the end users, target populations and policy makers, and taking corrective action if results don't meet expectations. In a well-managed organization, all staff members have two responsibilities – to do their job and to improve their work processes. It is upon the management to direct and support staff in realizing these responsibilities.

A health system is managed by a team and will be most productive and efficient if the team focuses on technique – the way the team members work together, the methods through which services are delivered. It is very easy to focus on *what* you do. It is more important to focus on *why* you do it. Today the paradigm of health care is becoming more client-centred, a concept that leads to demands for client-oriented, health promoting and higher quality health care services. Thus managers need to keep focused on the population for whom they are working to reduce suffering and mortality caused by disease. Continuous improvement is at the heart of process management.

THEORIES OF MANAGEMENT

There are various management theories as listed below:

- Classical management.
- Human Relations
- Situational/Contingency.
- Management by Objectives
- Systems approach to management
- Bureaucratic Management

Classical approach

This approach was developed between 1900 and 1910 and extended through to early 1950s. The theory had 3 branches. These are:

- Scientific Management
- Administrative/Principles of Management
- Bureaucratic Management

Scientific management theory

This is a technique of management whereby the work methods are scientifically studied and standards established. It is a philosophy about the relationship between the people and work - not technique or efficiency device.

The scientific Management approach was developed by an American engineer called Frederick Winslow Taylor, (1856 - 1915). He came up with this theory because the systematic approach (formally used) failed to lead to widespread production efficiency. He discovered that:

- Production and pay were poor.
- Inefficiency and waste were common.
- Many companies had unused potential.

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Principles of Scientific management theory
The scientific management theory has 4 principles. These are: -

- Developing scientific methods of designing jobs to replace the rule of thumb methods. This involves collecting and analyzing data to arrive at the best way of performing tasks or series of tasks.
- The Scientific selection and progressive teaching and developing employees to improve performance.
- Integrating scientifically selected employees and developed methods of designing jobs. The Scientific methods of job design should be well explained by management to reduce resistance.
- A division of work resulting in the interdependence between management and workers. Taylor believed that if they were truly dependent on one another, then co-operation would naturally follow.

NOTE:

1. Scientific management theory emphasized on specialization and division of labour.
2. Taylor and other scientific management pioneers believed that employees could be motivated by economic rewards, provided those rewards were related to individual performance. "The more you work the more you earn".

Limitations of scientific management
These are:

- Simplistic motivational assumptions.
- Workers were viewed as part of machine.
- Exploitation of labour.
- Ignored relationship between the organizations and its environment.
- Excluded senior management tasks.

These limitations prompted the development of administrative management theory.
Administrative/principles of management theory
It is also referred to as principles of management. This type of management places emphasis on the members of the organization usually referred to as "body corporate", that is, the personnel or employees.
The Administrative theory was developed by a French top Management specialist called **Henri Fayol**, (1841-1925). The theory emerged in 1916 and it emphasized the perspective of senior managers within the organization. Taylor argued that management was a profession and could be taught.
Fayol in his contribution, defined management as comprising of five (5) elements. These are:

- Planning
- Organizing
- Commanding
- Coordinating and
- Controlling

In addition, Fayol stated 14 principles that he wanted them applied flexibly. These are:

- **Division of work -** Divide work into specialized tasks (elements) and assign responsibility to specific individual.
- **Authority -** Delegate authority a long with responsibility (workers are given the right to command to get the work done).

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- **Discipline -** Rules and regulations should be enforced fairly and judiciously. There should be well- defined procedures of dealing with disciplinary matters.
- **Unity of command -** Each employee should be assigned to only one supervisor, otherwise conflict and confusion in authority and instructions would result.
- U**nity of direction -** The principle of Fayol states that "there should be one head and one plan "for group of activities/tasks having the same objectives. For example, the personnel department should have only one personnel officer with a specified plan for personnel policies, feeding personnel in all departments of the organization.
- **Subordination of individual interest to organization interest -** Interests of the organization must take priority over the interests of an individual or a group, whenever there is a conflict between the two.
- **Remuneration of employees -**All workers should be fairly paid with appropriate additional incentives for additional efforts.
- **Centralization -** All major policy formulation and decisions should be made at the top management level (central point).

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- **Scalar chain -** Communication should follow the formal channel in the hierarchy. Keep the communication within the chain of command.
- **Order -** Place for everything and everything in its place.
- **Equity -** Fair discipline and order enhance employee commitment. Managers should be fair and friendly to subordinates and they should exercise impartiality.
- **Initiative -** Encourage employees to act on their own in support of the organization's direction regardless of the mistakes made.
- **Esprit de corps**
  Promote a unity of interests between employees and management. Workers should work as a team.
- **Stability of staff/tenure of employees**
  Employee turnover should be minimized. Tenure and long term commitment should be encouraged. It results in a sense of belonging to organization giving rise to dedication and better outcome/results/output.

Although in his contributions, management was viewed as a profession that could be trained and developed, emphasized the broad policy aspect of top - level managers and offered universal management prescriptions, it had limitation: -

Limitation

The universal prescriptions need qualifications for environmental, technological and personnel factors.

Bureaucratic management theory

It was developed by a German Sociologist, Lawyer and Historian called Max Weber (1864 - 1924). He was interested in the work of Taylor. His theory was based on authority structures (hierarchy) of a form of an organization. He was specifically concerned with the administration, *that is, how the organizations are structured in terms of "who is responsible for who or to who along with the activities to be carried out*. He thought bureaucracy would be:

i) Fairer
ii) More impartial and
iii) More rational

Characteristics of bureaucratic theory

- Workers are selected and promoted on the basis of technical competence and experience (merit).
- A system of rules covering the duties and rights of employees that must be adhered to.
- Well defined hierarchy of authority.
- A division of labour by functional specialization.
- A system of procedures for dealing with work situations.
- Job- holders are expected to be impartial, that is, practice fairness and no favourism.

Limitations

The shortcomings to this theory are:

- It had limited organizational flexibility and it is slow in decision making.
- It ignores the importance of people and interpersonal relations.
- It accumulates power that could result in authoritarian management.

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- Rules may become ends in themselves.
- Difficult to dismantle once established.
- Too much specialization that could result, for example, patient to die in hospital if the person assigned to that responsibility is not there.

These limitations come because of the complexity of organizations. It is better for small organizations, although it is mainly applied in the government organizations.
Bureaucratic theory served to fit the traditional organizations which were less innovative in their operations and later around 1930's, human relations theory was developed to look at the human aspect of the organization.

Human relation theory

The theory was developed by Professor Elton Mayo around 1930s and it was backed up by A. Maslow, Mcgregor (1960), Hersberg, Likert, Argyris and Chester Bernard (1938).

Management Functions

Managerial work consists of a number of well-defined but interrelated functions (see Figure 1 below):

- ***Planning:*** Working in a broad outline of the things that need to be done and the methods for doing them to accomplish the purpose set for organization.
- ***Organizing:*** At the senior most level, establishing the formal structure of an organization through which work subdivisions are arranged, defined and coordinated for the whole organization; at lower levels carrying out similar functions for the specific department or division.
- ***Guiding:*** On an ongoing basis, making decisions, embodying them in specific general orders and instructions, and serving as a leader of the organization.
- ***Controlling:*** Coordinating, monitoring, evaluating, and supervising.

Management by Objectives

In this management approach the manager sets clear objectives to be realized. This is repeated in every department in the organization to help realize the broad organizational goal. As noted earlier in Chapter 4 on planning, objectives must be SMART so as to ensure the organization realizes its objectives. To recap, SMART refers to:

- ***Specific:*** The objective is exact, distinct and clearly stated. Vague language or generalities are not results. It identifies the nature of expected changes, the target, region, etc. It should be as detailed as possible without being wordy.
- ***Measurable:*** The objective is measurable qualitatively and/or quantitatively.
- ***Achievable:*** The objective is achievable with the human and financial resources available.
- ***Realistic:*** The objective addresses specific, recognized challenges and is within mandate.
- ***Time-bound:*** The objective is to be achieved in a stated time-frame or planning period

Principles of Management

Basic principles of management were set out by Henry Fayol (1841–1925), a French management theorist who developed one of the first comprehensive statements of a general theory of management.[1] Still relevant today, the 14 principles Fayol articulated are:

- ***Division of work*:** The specialization of the workforce according to the skills a person has, creating specific personal and professional development within the labour force and therefore increasing productivity; leads to specialization, which increases the efficiency of labour. By separating a small part of work, the workers' speed and accuracy in its performance increases. This principle is applicable to both technical and managerial work.

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- ***Authority and responsibility:*** The issue of commands followed by responsibility for their consequences. Authority means the right of a superior to give orders to subordinates; responsibility means obligation for performance. This principle suggests that there must be parity between authority and responsibility. They go together and are two sides of the same coin.
- ***Discipline:*** Discipline refers to obedience, proper conduct in relation to others, respect of authority, etc. It is essential for the smooth functioning of all organizations.
- ***Unity of command:*** This principle states that every subordinate should receive orders and be accountable to one and only one superior. If an employee receives orders from more than one superior, it is likely to create confusion and conflict. Unity of command also makes it easier to fi x responsibility for mistakes.
- ***Unity of direction:*** All those working in the same line of activity must understand and pursue the same objectives. All related activities should be put under *one group*, there should be *one plan* of action for them, and they should be under the control of *one manager*. Unity of action seeks to ensure a focus of efforts and coordination of strength.
- ***Subordination of individual interest:*** The management must put aside personal considerations and put company objectives first. Therefore the interests of the goals of the organization must prevail over the personal interests of individuals.
- ***Remuneration:*** Workers must be paid sufficiently as this is one of the chief motivations of employment and therefore greatly influences productivity. The quantum and methods of remuneration payable should be fair, reasonable and rewarding of effort.
- ***The degree of centralization:*** The amount of power wielded within the central management depends on company size. Centralization implies the concentration of decision making authority at the top management. Sharing of authority with lower levels is called decentralization. The organization should strive to achieve a proper balance.
- ***Scalar chain:*** Scalar chain refers to the chain of superiors ranging from top management to the lowest rank. The principle suggests that there should be a clear line of authority from top to bottom linking all managers at all levels. It is considered a chain of command. It involves a concept called a "gang plank", through which a subordinate may contact a superior or the superior's superior in case of an emergency, defying the hierarchy of control. However, the immediate superiors must be informed about the matter.
- ***Order:*** Social order ensures the fluid operation of a company through authoritative procedure. Material order ensures safety and efficiency in the workplace.
- ***Equity:*** Employees must be treated kindly, and justice must be enacted to ensure a just workplace. Managers should be fair and impartial when dealing with employees.
- ***Stability of tenure of personnel:*** The period of service should not be too short and employees should not be moved from positions frequently. An employee who is removed before becoming accustomed to the assigned work cannot render useful service.
- ***Initiative:*** Tapping into the initiative of employees can add strength and new ideas to an organization. Initiative on the part of employees is a source of strength for the organization because it provides new and better ideas. Employees who know their ideas are respected are likely to take greater interest in the functioning of the organization.
- ***Esprit de corps:*** This refers to the need of managers to develop and ensure morale in the workplace, individually and communally. Team spirit helps develop an atmosphere of mutual trust and understanding. These can be used to initiate and aid the processes of change, organization, decision making, skill management and the overall view of the management function.

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Managerial Roles
Managers clearly have many functions and roles. These can be summarized into three basic categories:

Interpersonal roles:
- Managers as leaders, providing guidance and motivation.
- Managers as liaison officers, maintaining a web of relationships with individuals and groups.
- Managers as disturbance handlers, deal with situations and change beyond their control

Informational roles:
- Managers as monitors, continually seeking and receiving information as a basis for action.
- Managers as disseminators, passing factual information to supervisors, colleagues and subordinates and transmitting value statements to guide subordinates in making decisions.
- Managers as spokespeople, transmitting information into their organization's environment.

Decision roles:
- Managers as entrepreneurs, acting as initiators of controlled change in the organization.
- Managers as resource allocators, making choices about scheduling their own time, authorizing actions and allocating people and finance to projects or activities.
- Managers as negotiators with other organizations or individuals.

Management Skills
For managers to be effective they require certain critical skills, including those in the list below. The skills may be used singly or in combination, depending on the situation and circumstances. They are:
- Computer skills
- Technical skills
- Human skills
- Communication skills
- Interpersonal skills

Leadership
Definition:
Leading is the process of influencing people, that is, personnel, followers so that they could contribute to the goals of the organization or group. Leadership on the other hand is the interpersonal influence, exercised in situations and directed through the communication process towards the organizational goals. It is concerned with guiding others or activities of followers/subordinates towards the pre-determined objectives/goals. A leader/manager must use influence (positive) to achieve the set objectives or goals.

However, there are as many definitions of leadership as there are people who have attempted to define it. One common theme that runs through each of them is the ability of an individual to inspire confidence and to provide support to members of a group to achieve the programme objectives.

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Leadership is a process or ability to influence the behaviour of others, to motivate and mobilize others to work together and achieve a common aim. It is a way of focusing and motivating a group of people to enable them to achieve their own aims and develop themselves. It also involves being accountable and responsible for the group as a whole. Leadership comprises three factors: the leader, the people, and the situation or conditions. Thus leadership is determined not only by the characteristics of the leader and the team, but also by the situations that prevail in the programme or organization.

Process of leading

Leading involves various activities and not merely leading people. It comprises of:

- Planning
- Organizing
- Directing
- Delegating
- Reporting
- Staffing and
- Motivating people

Are Leaders Born or Made?

- Some argue that leaders are born, while others hold that leaders are made. Whichever the case, the training of leaders – whether born or not – gives them skills that help them to become better leaders or managers.
- To maximize their potential individuals must begin to think of leadership in place of management, accountability in place of responsibility, job roles in place of job descriptions.
- To increase our leadership ability, we must expand ourselves. Leadership development is personal development. In order for us to understand ourselves, we must gather information; we must obtain the facts and data that tell us about ourselves. For individuals to maximize potential they must begin to think of leadership in place of management, of accountability in place of responsibility. They must begin to think of job roles in place of job descriptions
- Leadership is the art of inspiring followers to perform their duties willingly, competently and enthusiastically so as to achieve organizational objectives. Leadership is determined not only by the characteristics of the leader and the team, but also by the situations that prevail. A good leader has the ability to set direction and inspire others. The leader is willing to take risk, is creative and innovative, accountable and responsible. In a service-oriented organization, leadership determines the success or failure of the delivery of services. It provides inspiration and motivation for workers to perform.

Qualities of Good Leadership

These are numerous, as the following list suggests:

- ***Vision:*** Good leaders have a vision of the future and lead their people to their vision. A vision must be shared or it will be useless.
- ***Creativity:*** A good leader is creative – able to come up with new ideas and ways of doing things.
- ***Awareness:*** Good leaders have the ability to perceive opportunities in the environment within which they are operating, to enable them to utilize the context to achieve the intended objectives.

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- ***Initiative:*** This is exercised whenever effort is concentrated on a specific activity, to start something that was not going on before, to stop something that was occurring, or to shift the direction and character of the effort.
- ***Inquisitive:*** This is the ability to gain access to facts and data from people or other information sources. The quality of information may depend on a leader's thoroughness, keenness and commitment. A leader who is keen to learn as much as possible about work activities is more likely to gain quality information than one who ignores the need for enquiry.
- ***Supportive:*** A good leader is supportive to the led. The leader in return also gets support so that the vision and mission are realized.
- ***Advocacy:*** This means taking a position in support of a cause, e.g., creating awareness on cost sharing. A leader has the ability to convince or persuade, and is prepared to take a stand.
- ***Making decisions:*** This involves choosing or selecting from two or more courses of action. It may involve choosing an intervention or how best available resources can be effectively used.
- ***Critiquing:*** Good leaders are able to give (and accept) constructive critique and feedback. Management teams need to use the "Critique Approach" when conducting supervision, counselling and guidance of their subordinates.

Leadership Styles

It is generally accepted that leadership style – that is, the way leadership ability is exercised – can be grouped into fi ve[3] categories:

- ***Autocratic/dictatorship:*** Autocratic leaders use their legitimate authority to suppress their followers in order to realize their own goals.
- ***Democratic:*** Democratic leadership is about majority rule; the leader goes by the decision of the majority. The weakness of this type of leadership is that the minority are suppressed and their views are rarely heard.
- ***Laissez-faire:*** This is a leadership style in which the leader provides minimal direction and gives employees power to determine goals, make decisions and resolve problems on their own. This style can be effective when employees are highly skilled, experienced, self-motivated and well educated. Otherwise, it can result in chaos.
- ***Paternalistic:*** These leaders act as fathers to their followers.
- ***Charismatic leadership:*** The leader has a magnetic personality that in itself attracts people and instils loyalty. This type of leader is normally referred to as a born leader.

Leadership Skills

As might be surmised from the previous section, leadership involves the following important skills:

- Getting and giving information
- Understanding group needs and characteristics
- Knowing and understanding group resources
- Controlling the group
- Counselling
- Setting the example
- Representing the group
- Planning
- Evaluating – progress, performance, people
- Sharing leadership
- Managing learning

The Difference between Managers and Leaders

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It is important to understand the difference between management and leadership. While management deals with the administrative aspects of a manager's work (planning, organizing, monitoring), leadership deals with the interpersonal aspects (inspiration, motivation, influence).
Qualities of both a manager and a leader are necessary. While performing managerial or administrative roles (planning, organizing and controlling), managers are often called upon to take up leadership roles that deal with the interpersonal aspects of their job (inspiration, motivation, influence) that may have a long-lasting impact on individuals and the group.
A manager without leadership skills is an administrator, while a creative, far-sighted leader who is unable to implement a vision (without managerial skills) will confuse and lose the team. There may be only one or two designated leaders in an organization, such as the director or team leader, but leadership qualities can be found in many others within the team.
Clearly, the managers must perform both leadership and management functions. The major responsibilities of a manager are to operate and maintain the organization efficiently, ensuring that it provides useful services to clients and the community. Managers tend to be problem-solvers, seeking better ways to use their resources to get the job done. Effective managers bring order and consistency to programmes.
Leaders, on the other hand, are pathfinders; they branch out more in their thinking. They are concerned with building the organization for the future, providing direction, securing new resources, developing new capacities, preparing the organization to meet challenges and take advantage of emerging opportunities, adapting to change, and networking.

In summary:

- Management is a science; leadership is an art.
- Managers manage things; leaders lead people.
- Managers run institutions; leaders build institutions.
- Managers are busy; leaders make time.

Theories of leadership

Theories of leadership Interest in leadership increased during the early part of the twentieth century. Early leadership theories focused on what qualities distinguished between leaders and followers, while subsequent theories looked at other variables such as situational factors and skill levels. While many different leadership theories have emerged, most can be classified as one of eight major types:

1. Great Man Theories:

Great man theories assume that the capacity for leadership is inherent – that great leaders are born, not made. These theories often portray great leaders as heroic, mythic and destined to rise to leadership when needed. The term "Great Man" was used because, at the time, leadership was thought of primarily as a male quality, especially in terms of military leadership.

2. Trait Theories:

Similar in some ways to "Great Man" theories, trait theories assume that people inherit certain qualities and traits that make them better suited to leadership. Trait theories often identify particular personality or behavioral characteristics shared by leaders. If particular traits are key features of leadership, then how do we explain people who possess those qualities but are not leaders? This question is one of the difficulties in using trait theories to explain leadership.

3. Contingency Theories:

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Contingency theories of leadership focus on particular variables related to the environment that might determine which particular style of leadership is best suited for the situation. According to this theory, no leadership style is best in all situations. Success depends upon a number of variables, including the leadership style, qualities of the followers and aspects of the situation.

4. Situational Theories:

Situational theories propose that leaders choose the best course of action based upon situational variables. Different styles of leadership may be more appropriate for certain types of decision-making. For example, in a situation where the leader is the most knowledgeable and experienced member of a group, an authoritarian style might be most appropriate. In other instances where group members are skilled experts, a democratic style would be more effective.

5. Behavioral Theories:

Behavioral theories of leadership are based upon the belief that great leaders are made, not born. Rooted in behaviorism, this leadership theory focuses on the actions of leaders not on mental qualities or internal states. According to this theory, people can learn to become leaders through teaching and observation.

6. Participative Theories:

Participative leadership theories suggest that the ideal leadership style is one that takes the input of others into account. These leaders encourage participation and contributions from group members and help group members feel more relevant and committed to the decision-making process. In participative theories, however, the leader retains the right to allow the input of others.

7. Management Theories:

Management theories, also known as transactional theories, focus on the role of supervision, organization and group performance. These theories base leadership on a system of rewards and punishments. Managerial theories are often used in business; when employees are successful, they are rewarded; when they fail, they are reprimanded or punished**.**

8. Relationship Theories:

Relationship theories, also known as transformational theories, focus upon the connections formed between leaders and followers. Transformational leaders motivate and inspire people by helping group members see the importance and higher good of the task. These leaders are focused on the performance of group members, but also want each person to fulfill his or her potential. Leaders with this style often have high ethical and moral standards**.**

Change management

Managing Change

Organizations are designed to fit the environment in which they operate at a given time. Their environments are constantly changing in varying degrees. Change is therefore inevitable in any organization that seeks to maintain its relevance. Managers deal with many organized groups representing various interests. Most of these groups exert power over operations of organizations. While they may be able to do little or nothing to change these forces, it is nevertheless up to the manager to anticipate and integrate these various forces that are bound to affect his operations.

Different types of change require different strategies to implement the change successfully. Managers must anticipate changes in the environment that will require future adjustments in the organizations designs. Change management is a systematic approach to dealing with change, both from the perspective of an organization and at the level of the individual.

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Change management is a systematic approach to dealing with change, both from the perspective of an organization and on the individual level. Change management means to plan, initiate, realize, control and finally stabilize change processes, whether corporate or personal. Change may cover such diverse problems as, for example, strategic direction or personal development programmes for staff.

The Importance of Change Management

The capacity to manage change is the most important characteristic of leaders who succeed. This capacity depends on seven essential abilities:

- To be capable of adapting to the environment and be conscious of the gap between what is done and what can be done; to appreciate but never be satisfied with the situation as it is today and pay more attention to what does not function than to what functions.
- To be able to question the present reality.
- To be able to communicate a compelling vision of the direction the organization must take.
- To have the capacity to create a coalition of followers and supporters.
- To be capable of building a team capable of making change happen.
- To persist and persevere.
- To share with others the credit and recognition brought by success.

Types of Change

Change is the continuous adaptation of corporate strategies and structures to changing external conditions. It may take several forms, as described below:

Developmental Change Developmental change occurs when an organization makes an improvement to its current way of doing business. If an organization decided to improve its processes, methods or performance standards, this would be considered developmental change.

Transformational Change Transformational change occurs after the transition period. Transformational change may involve both developmental and transitional change, and it is common for the two types to occur in tandem. This happens mainly when organizations are faced with the emergence of radically different technologies, significant changes in supply and demand, unexpected competition, lack of revenue, or other major shifts in how they do business. Formal changes are the responses of an organization to major internal forces and external pressures (change drivers).

Transitional Change

Transitional change is more intrusive than developmental change, as it replaces existing processes or procedures with something that is completely new to the organization. The period when the old process is being dismantled and the new process is being implemented is called the transitional phase.

Guidelines for Change Management

Resistance to change can be avoided or reduced by doing the following:

1.) Planning change carefully:

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- Identify problems and their underlying causes.
- Formulate alternative actions to solve the problems.

Select the most feasible action(s).

- Establish an implementation plan.

2.) Involving those who will be affected by change right from the planning stage:

- Explain the benefits, for both the organization and the people, and also indicate the disadvantages that are likely to arise.
- Introduce change gradually.
- Be patient and tolerant during change; give support and help.
- Provide training that may be needed.
- Monitor change.
- Communicate frequently with people during and after change in order to identify snags and problems.
- Be firm about the end results but flexible about getting there.

Kurt Lewin's Change Model

An early model of change developed by Kurt Lewin (1890–1947),[4] one of the modern pioneers of social, organizational and applied psychology, described change as a three-stage process:

1.) ***"Unfreezing":*** It involves overcoming inertia and dismantling the existing "mind set". Defence mechanisms have to be bypassed.

2.) *"**Change":*** This is typically a period of confusion and transition. We are aware that the old ways are being challenged, but we do not yet have a clear picture of what we are replacing them with.

3.) ***"Refreezing"***: The new mind set is crystallizing and one's comfort level is returning to previous levels. This is often misquoted as "refreezing" (Lewin, 1947).

Note: Change is inevitable in the current complex environment. It has been said that change is the only constant. A leader would learn change practices that produce results.

Resistance to change

Change is actually a frightening word for many people. People resist and try to avoid changes that will leave them worse off than they are now. But, resistance may be met even if the proposed change is neutral, or beneficial to the persons concerned.

Why people resist change

- They believe that is not *in their interest* to change
- They think the change is the *wrong thing* to do, if you knew what I knew"
- They are *skeptical* about the possibility of success
- They *don't understand why* change is happening
- It isn't relevant to them
- They don't understand exactly *what change* is required
- They don't have the *time or resources* to change
- They don't think that they will be able to change *(Fear of failure)*
- They feel they were not involved in the decision to change
- Lack of conviction that change is needed
- Dislike of imposed change
- Dislike of surprises
- Fear of the unknown

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- Reluctance to deal with unpopular issues fear of
- inadequacy and failure
- Disturbed practices, habits and relations
- Lack of respect and trust in the person promoting change

Strategies for dealing with change resistance

1. Education & Communication:

One of the best ways to overcome resistance to change is to educate people about the change effort beforehand. Up-front communication and education helps employees see the logic in the change effort. This reduces unfounded and incorrect rumors concerning the effects of change in the organization

2. Participation & Involvement:

When employees are involved in the change effort they are more likely to buy into change rather than resist it. This approach is likely to lower resistance more so than merely hoping people will acquiesce to change

3. Facilitation & Support:

Managers can head-off potential resistance by being supportive of employees during difficult times. Managerial support helps employees deal with fear and anxiety during a transition period. This approach is concerned with provision of special training, counseling, time off work.

4. Negotiation and Agreement:

Managers can combat resistance by offering incentives to employees not to resist change. This can be done by allowing change resistors to veto elements of change that are threatening, or change resistors can be offered incentives to go elsewhere in the company in order to avoid having to experience the change effort. This approach will be appropriate where those resisting change are in a position of power.

5. Manipulation and Cooptation:

"Cooptation" (no it's not misspelled) involves the patronizing gesture of bringing a person into a change management planning group for the sake of appearances rather than their substantive contribution. This often involves selecting leaders of the resisters to participate in the change effort. These leaders can be given a symbolic role in decision making without threatening the change effort.

6. Explicit and Implicit Coercion:

Managers can explicitly or implicitly force employees into accepting change by making clear that resisting change can lead to losing jobs, firing, or not promoting employees

Group Dynamics

Group dynamics refers to a system of behaviors and psychological processes occurring within a social group (*intra*group dynamics), or between social groups (*inter*group dynamics). The study of group dynamics can be useful in understanding decision-making behavior, tracking the spread of diseases in society, creating effective therapy techniques, and following the emergence and popularity of new ideas and technologies. Group dynamics are at the core of understanding racism, sexism, and other forms of social prejudice and discrimination. These applications of the field are studied in psychology, sociology, anthropology, political science, epidemiology, education, social work, business, and communication studies.

Team building

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Team building is a philosophy of job design in which employees are viewed as members of interdependent teams instead of as individual workers. Team building refers to a wide range of activities, presented to businesses, schools, sports teams, religious or nonprofit organizations designed for improving team performance.

Team building is pursued via a variety of practices, and can range from simple bonding exercises to complex simulations and multi-day team building retreats designed to develop a team (including group assessment and group-dynamic games), usually falling somewhere in between. It generally sits within the theory and practice of organizational development, but can also be applied to sports teams, school groups, and other contexts.

Team building is not to be confused with "team recreation" that consists of activities for teams that are strictly recreational. Team building can also be seen in day-to-day operations of an organization and team dynamic can be improved through successful leadership. Team building is said to have benefits of self-development, positive communication, leadership skills and the ability to work closely together as a team to solve problems.

Work environments tend to focus on individuals and personal goals, with reward & recognition singling out the achievements of individual employees. Team building can also refer to the process of selecting or creating a team from scratch

Team dynamic

When assembling a team it is very important to consider the overall dynamic of the team. When building a team, five dynamics are fundamental to team success:

1. **The team member**: Successful teams are made up of a collection of effective individuals. These are people who are experienced, have problem solving ability, are open to addressing the problem, and are action oriented.
2. **Team relationships**: For a team to be successful the members of the team must be able to give and receive feedback.
3. **Team problem solving**: An effective team depends on how focused and clear the goal of the team is. A relaxed, comfortable and accepting environment and finally, open and honest communication are required.
4. **Team leadership**: Effective team leadership depends on leadership competencies. A competent leader is: focused on the goal, ensures a collaborative climate, builds confidence of team members, sets priorities, demonstrates sufficient “know-how” and manages performance through feedback.
5. **Organizational environment**: The climate and culture of the organization must be conducive to team behavior. Competitiveness should be discouraged and uniformity should be encouraged - this will eliminate conflict and discord among team members.

Goals

The overall goals of team building are to increase the teams understanding of team dynamics and improve how the team works together. Unlike working as a group, working as a team incorporates group accountability rather than individual accountability and results in a collective work product. Team building encourages the team approach to working on a project. Advantages to this approach include:

- Increased flexibility in skills and abilities
- More productive than work groups with individual mindset

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- More beneficial in times of organizational change
- Encourage both individual and team development and improvement
- Focuses on group goals to accomplish more beneficial tasks
- Improved range of team building objectives such as collaboration, communication and increased creative or flexible thinking.

Eight (8) effective strategies a leader should employ to enhance team building:

1. Establish clear and inspiring team goals
2. Maintain a results-oriented team structure
3. Assemble competent team members
4. Strive for unified commitment
5. Provide a collaborative climate
6. Encourage standards of excellence
7. Furnish external support and recognition
8. Apply principled leadership

Organizational mission, vision and core values

Vision:

This involves organization developing a clear understanding of what will be its preferred future. This might not be difficult for organizations who have been managing using strategic plan since they can adopt what they have or modify to suit the ever changing trends in technology and innovations in management approaches. To achieve the vision operation plans have to be developed.

Mission

A mission is at a core of why organization do the work they do. This involves developing sound statement about why an organization exists. It reflects purpose and may include some strategy reflection. In health Records and Information Department for example, they have the training mission statement as "To train competent Health Records and Information Technicians who will provide quality health care to the community". However, if and organization do not existing one.

Core values and beliefs

Values are beliefs that are shared among the stakeholders of the organization and form the principles or commitments that the organization members stand for. This is where the behaviors and ideas that are important to an organization are described. The examples are the respect of staff is the most valuable resource in an organization, Professionalism, integrity and ethic, a conducive working and learning environment, creativity, inquisitiveness and teamwork, equity across different individuals and /or groups in society and Partnerships with stakeholders.

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ORGANIZATION OF HEALTH SERVICES

At the end of this unit, the learning should be able to:

1. Describe the organizational structure of the Health care system

Organization Structure

Kenya's health system has undergone significant change and reform in recent years and with the promulgation of the 2010 Constitution will continue to evolve. Even so, the system will reflect the inter-linked levels of national, provincial, district, sub-district and community. (These levels of local administration are subject to change in line with the implementation of the new Constitution of Kenya 2010, which provides for devolution of both authority and financial resources). The national level is made up of the MOMS and MOPHS headquarters, with the provincial level serving as de-concentrated units of the national level. (Figures 3.1 and 3.2 outline the organization of the Ministry of Medical Services and the Ministry Public Health and Sanitation, respectively.)

The government health service is supplemented by privately owned and operated hospitals and clinics and faith-based organisations' hospitals and clinics, which together provide between 30 and 40 percent of the hospital beds in Kenya.

The functions of the national level are:

1. Strategic planning and policy formulation,
2. Enforcement of standards,
3. Ensuring commodity security,
4. Performance monitoring,
5. Capacity strengthening,
6. Resource mobilization, and
7. Operational and other research

The oversight and policy formulation functions are managed through the Ministers and Assistant Ministers. The Permanent Secretary (PS) is the link between the technical, administrative and oversight functions, while the Director of Medical Services (DMS) and the Director of Public Health and Sanitation (DPHS) are the coordinators of the technical functions of the respective ministries. The DMS and the DPHS execute their functions through the departments in the respective ministries. These departments are further divided into divisions as illustrated in the figures below.

Purpose of Health structures

The purpose of an organizational structure is to define the guidelines, parameters and the procedural process necessary for a group to accomplish a main objective. For example, the anatomy of an organizational structure is further reduced to the distribution of authority, span-of-control, and line vs. staff structures, organizational height and departmentalization. An organizational structure organizes priorities hierarchically by means of identifying tasks

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critical to a group realizing an end goal. An organizational structure may adopt a live structure or a staff structure or both to achieve their main objectives. A line structure, sometimes called a product structure, identifies the activities directly responsible for the organization's main goal, such as the labor involved in making an actually product. A staff structure is the support staff or network assisting the line structures in their goals.

Types of health Structures

Functions of Health Structures
Organizational structure of the Health care systems

The Health Care System
The health sector comprises the public system, with major players including the MOH and parastatal organisations, and the private sector, which includes private for-profit, NGO, and FBO facilities. Health services are provided through a network of over 4,700 health facilities countrywide, with the public sector system accounting for about 51 percent of these facilities.

The public health system consists of the following levels of health facilities: national referral hospitals, provincial general hospitals, district hospitals, health centres, and dispensaries.

National referral hospitals are at the apex of the health care system, providing sophisticated diagnostic, therapeutic, and rehabilitative services. The two national referral hospitals are Kenyatta National Hospital in Nairobi and Moi Referral and Teaching Hospital in Eldoret. The equivalent private referral hospitals are Nairobi Hospital and Aga Khan Hospital in Nairobi.

Provincial hospitals act as referral hospitals to their district hospitals. They also provide very specialized care. The provincial level acts as an intermediary between the national central level and the districts. They oversee the implementation of health policy at the district level, maintain quality standards, and coordinate and control all district health activities. Similar private hospitals at the provincial level include Aga Khan Hospitals in Kisumu and Mombasa.

District hospitals concentrate on the delivery of health care services and generate their own expenditure plans and budget requirements based on guidelines from headquarters through the provinces.

The network of *health centres* provides many of the ambulatory health services. Health centres generally offer preventive and curative services, mostly adapted to local needs.
Dispensaries are meant to be the system's first line of contact with patients, but in some areas, health centres or even hospitals are effectively the first points of contact. Dispensaries provide wider coverage for preventive health measures, which is a primary goal of the health policy.

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- Kenya's health care provision and implementation infrastructure include the national teaching hospital, provincial hospitals, district and sub district hospitals, health centres, and dispensaries, community health services
- There is also a host of other operators within the private, non-governmental, and traditional/informal sectors.
- The system is a hierarchical-pyramidal organization comprising five levels, the lowest being the village dispensary and the Kenyatta National Hospital at the apex.
- The mandate for supervision, formulation of policies, establishment and enforcement of standards, and mobilization of resources for health care rests with the Ministry of Health
- The country has eight provinces divided into lower levels of administrations called districts, which are responsible for delivering health services and implementing health programs.
- Under the decentralization strategy, districts form the central pillars of the public health system.
- Management of healthcare at the district level is headed by a **District Medical Officer of Health (DMOH)** appointed by the Ministry of Health.
- The DMOH is supported by a District Health Management Board (DHMB) comprising officials appointed by the MOH and from local areas, and a professional unit, the District Health Management Team (DHMT).
- The DHMT prepares technical advisories and the District Health Plan in consultation with local health actors and the DHMB.
- According to the most recent health management information system (HMIS) data, there are over 9470 health facilities across the country operated by three owner systems.
- The government running 43% of the facilities, non-governmental organizations (NGOs) 3.4%, and private businesses 37% all other ownership is 16.6%.
- The government owns most of the hospitals, health centres, and dispensaries, while clinics and nursing homes are entirely in the hands of the private sector.

Structures

Health service delivery

Levels of service and Health services at each level

- Six levels of service 1-6

Level 6 National referral hospitals.

- are at the apex of the health care system, providing sophisticated diagnostic, therapeutic, and rehabilitative services.
- The two national referral hospitals are Kenyatta National Hospital in Nairobi and Moi Referral and Teaching Hospital in Eldoret.
- The equivalent private referral hospitals are Nairobi Hospital and Aga Khan Hospital in Nairobi.

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- They are centres of excellence and provide complex health care requiring more complex technology and highly skilled personnel.
- They have a high concentration of resources and are relatively expensive to run.
- They also support the training of health workers at both pre-service and in-service levels.

Teaching and referral hospitals have the following functions:

- **Health care.** Provide complex curative tertiary care. They also provide preventive care and participate in public health programmes for the local community and the total primary health care system
- **Quality of care**. Teaching hospitals should provide leadership in setting high clinical standards and treatment protocols. The best quality of care in the country should be found at teaching and referral hospitals.
- **Access to care**. Patients may only have access to tertiary care through a well-developed referral system
- **Research.** With their concentration of resources and personnel, they contribute in providing solutions to local and national health problems through research, as well as contributing to policy formulation
- **Teaching and training.** Teaching is one of the primary functions of these hospitals. They provide both basic and post-graduate training for health professionals.

Level 5 Provincial hospitals

- Act as referral hospitals to their district hospitals.
- They also provide very specialized care. The provincial level acts as an intermediary between the national central level and the districts.
- They oversee the implementation of health policy at the district level, maintain quality standards, and coordinate and control all district health activities.
- Similar private hospitals at the provincial level include Aga Khan Hospitals in Kisumu and Mombasa
- Their personnel include medical professionals, such as general surgeons, general medical physicians, paediatricians, general and specialized nurses, midwives, and public health staff

Provincial hospitals should provide clinical services in the following disciplines:

- Medicine;
- General surgery and anaesthesia;
- Paediatrics;
- Obstetrics and gynaecology;
- Dental services;
- Psychiatry;
- Accident and emergency services;
- Ear, nose and throat;
- Ophthalmology;
- Dermatology;
- ICU (intensive care unit) and HDU (high dependency unit) services.

They should also provide the following services:

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- Laboratory and diagnostic techniques for referrals from the lower levels of the health care system;
- Teaching and training for health care personnel such as nurses and medical officer interns;
- Supervision and monitoring of district hospital activities;
- Technical support to district hospitals such as specific outreach services.

Level 4 District Hospitals

Concentrate on the delivery of health care services and generate their own expenditure plans and budget requirements based on guidelines from headquarters through the provinces.

A district hospital should provide the following:

- Curative and preventive care and promotion of health of the people in the district;
- Quality clinical care by a more skilled and competent staff than those of the health centres and dispensaries;
- Treatment techniques such as surgery not available at health centres;
- Laboratory and other diagnostic techniques appropriate to the medical, surgical, and outpatient activities of the district hospital;
- They provide a wider range of services, such as basic curative and preventive services for adults and children, as well as reproductive health services.
- They also provide minor surgical services such as incision and drainage.
- They augment their service coverage with outreach services, and refer severe and complicated conditions to the appropriate level, such as the district hospital
- Inpatient care until the patient can go home or back to the health centre;
- Training and technical supervision to health centres, as well as resource centre for health centres at each district hospital;
- Twenty-four hour services;
- The following clinical services:
- Obstetrics and gynaecology;
- Child health;
- Medicine;
- Surgery, including anaesthesia
- Accident and emergency services;
- Non-clinical support services;
- Referral services;
- Contribution to the district-wide information generation, collection planning, implementation and evaluation of health service programmes.

Level 3 Health Centres

- Provides many of the ambulatory health services.
- Health centres generally offer preventive and curative services, mostly adapted to local needs.
- Health centres are staffed by midwives or nurses, clinical officers, and occasionally by doctors.

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- They provide a wider range of services, such as basic curative and preventive services for adults and children, as well as reproductive health services.
- They also provide minor surgical services such as incision and drainage.
- They augment their service coverage with outreach services, and refer severe and complicated conditions to the appropriate level, such as the district hospital

Level 2 Dispensaries

- Are meant to be the system's first line of contact with patients, but in some areas, health centres or even hospitals are effectively the first points of contact.
- Dispensaries provide wider coverage for preventive health measures, which is a primary goal of the health policy
- The dispensaries are at the lowest level of the public health system and are the first point of contact with patients.
- They are staffed by enrolled nurses, public health technicians, and dressers (medical assistants).
- The enrolled nurses provide antenatal care and treatment for simple medical problems during pregnancy such as anaemia, and occasionally conduct normal deliveries.
- Enrolled nurses also provide basic outpatient curative care

Level 1 Community level health services

- Network of simple or community-based health care programmes.
- Some sort of community committees is established.
- They may be specific health committees, part of development committee, parent-teacher association or some local group
- A person is then selected to attend a simple training course that prepares them to deal with health problems
- They are given a limited No. Of drugs or no drugs but refer people to dispensaries.
- May have like 50 households (250people) or more to look after
- These worker are called community health workers (CHWs), Village health workers (VHWs), helpers, scouts or motivators.

    - Four tiers

Health services at each level

Actors cadre

Health systems: main actors in the arena

Core partners have the following primary roles:

Funders, providers, purchasers, implementers, watchdogs

- Government

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- Private /NGOs
- Not for profit (e.g. faith-based)
- Private for profit
- Civil society organizations
- Development partners
- International
- Bilateral
- Foundations
- Media

Referral system in Kenya

For ease of reference and use, the guidelines are presented in 3 volumes:

Volume 1: Management Guidelines for Level 1 (Community)
Volume 2: Management Guidelines for Levels 2 and 3 (Primary Care)
Volume 3: Management Guidelines for Levels 4–6 (Hospitals)

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COMMUNICATION SKILLS

Introduction

Objectives;

By the end of this unit, the learner should be able to;-

- Define communication
- Explain the Communication process
- Outline the channels of communication
- Discuss the barriers to effective communication
- Explain strategies for overcoming communication barriers
- Describe how to listen and question effectively
- Discuss how to give feedback effectively
- Explain the Direction of organizational communication
- Discuss how to conduct and manage a meeting effectively.
- Discuss how to manage official correspondence effectively
- Explain the role of health managers in Customer Relationship Management (CRM)

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message that they interfere with the communication process.

- ***Ourselves:*** Focusing on ourselves, rather than the other person, can lead to confusion and conflict. Some of the factors that cause this are defensiveness (we feel someone is attacking us), superiority (we feel we know more than the other) and ego (we feel we are the centre of the activity).
- ***Perception:*** Our preconceived attitudes affect our ability to listen. We listen uncritically to persons of high status and dismiss those of low status. If we feel the person is talking too fast, or not fluently, or does not articulate clearly, or is speaking with an undesirable accent, etc., we may dismiss the person out of hand.

Overcoming Communication Barriers

As further illustrated in Figure 5.7, ways of minimizing communication barriers include:

- The use of different communication systems, including formal and informal methods, in order to facilitate understanding between the sender and the receiver of the message within an organization.
- The active role of managers in communication, for example in resolving conflict (relationship, task, social) and encouraging shared vision and team building.

Overcoming communication barriers; –

Becoming an active listener

1. Stop talking

Often, we talk more than we should without giving the other person a chance to respond. If we are thinking about what we will say when we talk, we cannot focus attention on the person we wish to listen to. Do not interrupt.

2. Pay attention

Do not allow yourself to be distracted by thinking about something else. Often, we need to make an active effort to pay attention when others are speaking.

3. Listen emphathetically

Try to take the speaker's perspective. Mirror the speaker's body language and give the person non-judgemental encouragement to speak.

4. Hear before evaluating

Do not draw premature conclusions or look for points of disagreement. Listen

- ***Clarity in communication:*** It is important to be clear about the subject and stick to the main theme of the topic when explaining it to someone else. It is also important to be well prepared yourself before trying to communicate with another person.
- ***Establish rapport:*** Because communication is an ongoing process that involves the active participiption of all concerned, people communicate effectively when they are at ease with one another. It is important to show interest in the other person, as this is the best way of getting their attention and acceptance.
- ***Encourage audience participation:*** This is especially important when unfamiliar or difficult issues are being discussed and collective decisions sought.
- ***Avoid ambiguity:*** The lack of specificity is one of the barriers to good communication. To avoid this it is necessary to be very clear when giving instructions or explanations. Vague statements such as "take appropriate action" or "make suitable arrangements" should be avoided; rather, the instructions should state exactly what needs to be done.
- ***Be brief:*** It is best to be direct and to the point and to reinforce the main points with

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examples if necessary.

Language: Special terms that may be unfamiliar to the listener should be explained and jargon avoided. If the language being used to communicate is a foreign or second language, it is best to use simple words and sentences, and to verify understanding.

- ***Develop listening skills:*** Since communication is a two-way process, it is as important to listen carefully as it is to speak properly. Sometimes we do not listen well for a number of reasons, such as boredom, bias, fear of being influenced and interruptions.
- ***Feedback:*** Feedback from the listener needs to be recognized by the sender, even when it is non-verbal (example yawning, being distracted, checking text messages). If what you are saying is not being registered or has the wrong effect, change your communication approach.
- ***Timing and situation:*** Effective communications requires enough time to cover the subject adequately without unnecessary interruptions. If interruption is unavoidable, it is important to set a formal date for continuing the discussion in the not too distant future.
- ***Verification:*** This goes back to the definition of communication as a two-way process involving both transmission and reception. To ensure accurate communication, it is important to ask questions to verify that your communication has been received in the way it was intended and that you heard the message that was intended.
- ***Use of different communication methods:*** A mix of formal and informal methods may facilitate understanding between the sender and the receiver of the message within an organization.

Role of effective communication

- Promotes teamwork,
- Enhances managerial and leadership skills,
- Enhances personal effectiveness,

(Involve the learner in adding more roles by brainstorming)

Channels of Communication

Channels of communication are mechanisms for linking one person to the other/others and vice versa. The channels vary, but those most commonly used to communicate with staff may be divided into organizational and interpersonal.

Organizational

Organizational communication is the consideration, analysis and criticism of the role of communication in organizational contexts; it may be formal or informal. For example, written communication becomes important when certain proceedings or instructions need to be recorded and guidelines/procedures need to be followed. Staff meetings may be called to give staff an opportunity to exchange information or solve problems, or to give instructions about a new skill that may need a detailed explanation or demonstration

Interpersonal

This is the process of face-to-face communication between individuals. Such communication may take several forms. Messages may be verbal (that is, expressed in words), or they may not involve words at all but consist of gestures, facial expressions and certain postures. For example, managers often communicate verbally with staff members on a one-to-one basis to

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discuss good or poor performance, to motivate and counsel them, to discuss and solve problems arising at the workplace, and to understand better the individual problems of their staff. They may also communicate in writing, to give instructions, get suggestions and document actions for disciplinary purposes.

Communication Networks

A network consists of a set of interconnected devices, nodes, hosts or computers linked across a communication medium. Communication networks, specifically, are links and nodes especially set so that messages may be passed from one part of the network to another through multiple links and various nodes. A network can offer a choice of services including voice transfer, fi le transfer, remote login and message passing. Therefore, networks, or patterns, are aspects of direction and flow of communication.

Direction of Organizational Communication

There are basically three directions in which communication operates – up, down and sideways:

- ***Downward communication:*** From supervisor to subordinate (e.g., job instructions, information on organization policies, performance feedback, information about the organization's goals and changes).
- ***Upward communication:*** From subordinate to supervisor (e.g., grievance procedures, department meetings, participation in decisions, others).
- ***Horizontal communication:*** This is communication between/among staff at the same level.

Ways to Improve Communication

Once you are aware of all the aspects influencing effective communication, you can begin to enhance your own communication skills. The following suggestions intend to help you to improve your skills in conveying – and receiving – messages.

Asking Good Questions

- Asking good questions is an art that can be learned
- Not all questions are good questions
- Sometimes they are statements or commands that are disguised as a question for example, "Don't you think you should have done that differently?"
- Such inquiry blocks learning
- Leading questions also block learning

Communication skills --Good questioning

Inquiry That Encourages Learning	**Inquiry That Blocks Learning**
How do you see this differently?	**Don't you agree? (especially when said in an intimidating way)**
What's your reaction to ?	**Did you do that because of X, Y, or Z?**
What led you to that conclusion/action?	**Do you really think you did a good job? (when you think he did not)**
Could you say more about that?	**Why don't you just try what I'm suggesting?**

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Why is that so?	**Why didn't you just tell me?**
What makes you ?	**Why are you so defensive?**
What kept you from telling me?	**Why don't you ?**
How do you think I/you contributed to that?	**What's the matter with you?**
How can I/you/we ?	**Don't you agree? (especially when said in an intimidating way)**

Effective Listening

- Good listening involves an attitude as well as a skill
- You have to believe that the other person is worth listening to, has something of value to say.
- Listening is difficult when your own mind is busy chattering away or occupied with concerns that have nothing to do with the other.
- Listening is hard when you have a strong urge to say something or when you have already come to a conclusion about the other person's problems or issues.
- There are three disciplines for learning to listen better.
  -focus:
  -empathy
  -silence
- To practice **focus,** periodically summarizing, briefly, what the other person said
- Ask if that is what she meant to say
- To practice **empathy,** try to rephrase in your own words what you think the other person must be feeling
- To practice **silence,** give time to the other person to think about a question you made.
- Practice being silence by not jumping in, counting until 30 if you must, and observe what happens.

FEEDBACK & SPEAKING HINTS

FEEDBACK **SPEAKING HINTS**

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• Evaluative • Interpretive • Supportive • Probing • Understanding	• Ask listener if they are following your message • Give receiver chance to comment or ask questions • Clarify and look at receiver • Ensure words match message, tone and body language • Vary tone and pace • Avoid ambiguity and note signs of confusion

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PUBLIC SPEAKING

Effective Presentations Skills

FIVE STEPS TOWARDS A GOOD PRESENTATION

1. **Decide what you want to achieve. what is the message?**
2. **Decide what form the presentation should take**
3. **Prepare a script**
4. **Rehearse it**
5. **Deliver it effectively**

Step 1. Decide what you want to achieve

- **What are you trying to achieve?**
- **Which three messages do you want your audience to take back with them?**
- **What are the arguments of the three messages?**
- **What is your flow?**

- **Identify arena**
  Nature of event, number of participants, time of day, time available, shape of room
- **Know your audience**

Who are they? What do they know already? How much can you take for granted? Will they understand the jargon?

Step 2. Form of the presentation

Will they understand the jargon?

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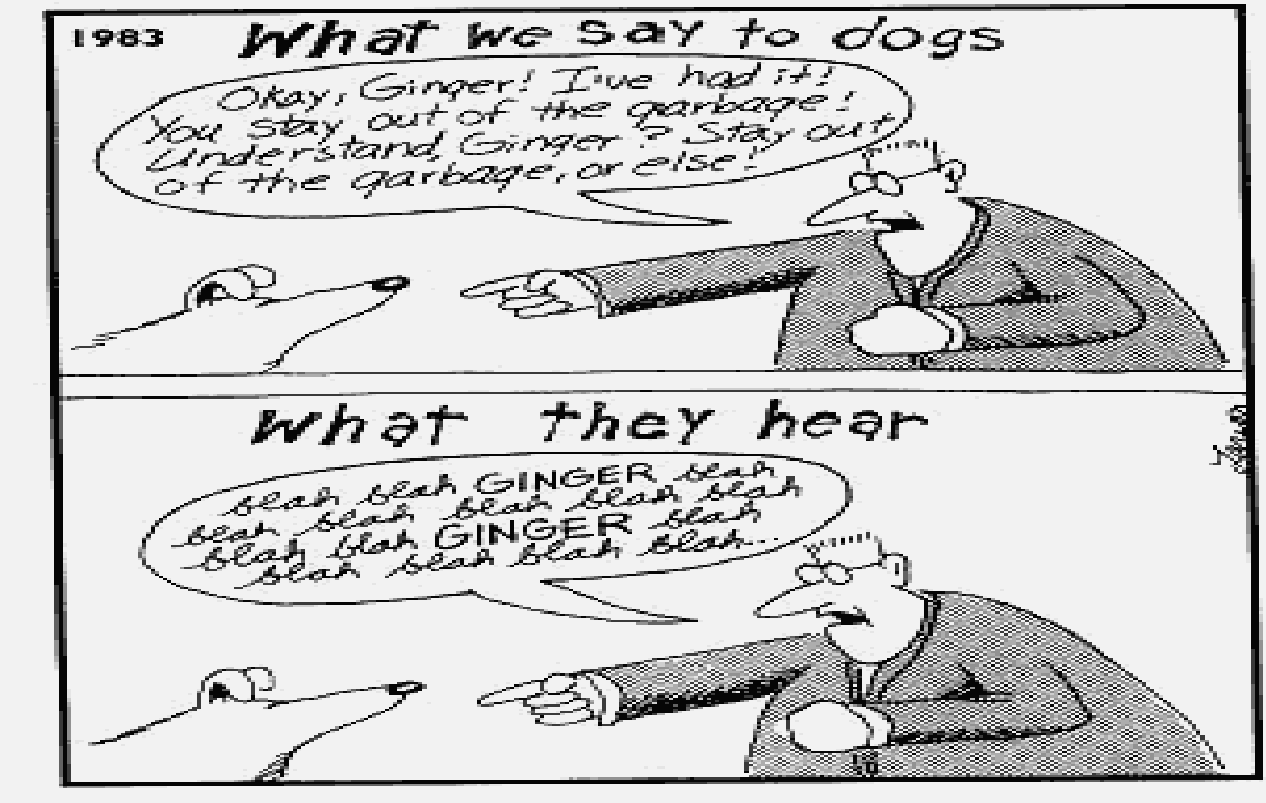

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Step3 Prepare a script (1)

How long?

Average attention curve max 25 to 40 minutes
Do you intend to use slides?

About 2 to 3 minutes per slide
How much new information?

20 minutes: 2 points - 30 minutes: 3 points
Written speech or cues?

Step 3: Prepare a script (2)

Structuring the content

Tell them what you are going to tell them. Tell them. Then tell them what you have just told them.

Step 4: Preparing for your presentation

- Rehearse:
    - How long does it really take?
    - How effective is it?
    - How good are my script and visual aids?
- Get a good colleague to listen to you
- Use your aids
- Go through the whole thing
- Debrief
- If you can, do it again!
- Clothes

Before the presentation

- Take a timeout
- Check your appearance
- Run cold water on hands
- Yawn, stretch, make faces, blow like a horse
- Shake your hands

Remember: You are the person who knows most about what you are going to say!

Step 5: Opening

- Find friends – about three or four will do
- The first minute is critical:

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- Know by heart the first two minutes of your presentation.

- Using hooks or ice breakers – all have pros and cons
  - Quotes
  - Humour
  - Rhetorical questions
  - Personal anecdotes
  - Shocking statistics
  - Outrageous statements

Step 5; Nonverbal behaviour

Non-verbal aspects of speech
Tone and colour of the voice
Volume
Rhythm/Pauses
Pronunciation/diction
Eye contact and body
Eye contact
Arms/gestures
Body language

Step 5: Closing

Vital part of the presentation

- **Common mistakes**
  - Cliché
  - Emergency stop
  - Endless maze or the more the merrier
- **Consider using "closing summary"**
- **When you come to the end stop!**

Visual aids

What we remember:

20% of what we hear
30% of what we see
50% of what we hear and see

- They should add to the presentation, not distract from it!
- They must be clearly visible and understandable!
- Speak to you audience – not to your visual aids
- Handouts before or after?

GOOD AND READABLE SLIDES

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Good slides

- Five to six bullets at the most
- Max 6 seconds to read
- Use charts or figures where possible
- Use large simple fonts (18pt or more)
- Use colours (but with caution!)

HEALTH ECONOMICS

Discussion questions

- What is health and health care?
- What is health economics?
- Why is it important?
- What makes the health care market different from the market or other goods?

What type of questions do health economists ask?

What is health?

Health is a multifaceted concept and not easily measurable. WHO defined as definition a state of complete physical and mental well-being and not merely the absence of disease or infirmity (WHO, 1948). It refer to peoples' health status (how healthy they are).

Important part of human capital:

- Human capital: value of learning, experience and ability embodied in workers which increases productivity and income.
- Asset: accumulates and depreciates

Individual or households can improve their health through use of health care, diet ..

Production of health:

- Health Production Functions
- Determinants of health

What is health care?

Definition:

It is the prevention, treatment, and management of illness and the preservation of mental and physical well-being through the services offered by the medical and allied health professions.

Important difference between health and health care is that, health care can be traded on the market but health cannot.

- Demand health care to improve our health
  - Demand for Health Care
- Health care markets differ from markets for other commodities

Roots of health economics:

- Concerned with the health market not with health or health status.

What is health economics?

Economics is the study of how people allocate their limited resources in an attempt to satisfy their unlimited wants. Health economics on the other hand is the study of how (scarce) resources are allocated to and within the health economy.

Basic concepts of supply and demand analysis

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Definition of Demand: It is the need and ability and willingness to pay for a commodity - The schedule of amounts of any product that buyer will purchase at different prices during some Stated time period.

Desire: refers to people's willingness to own a good. Demand is the amount of a good that consumers are willing and able to buy at a given price.

Definition Supply: the amounts of a good producer are willing and able to sell at a given price.
For many people the word market conjures up a picture of a town square with lots of small stall holders selling everything from fruit and vegetables to meat and fish. For economists,
the term has a much wider meaning. It is used to describe any process of exchange between buyers and sellers.

Formally, a market can be defined as any set of arrangements which allows buyers and sellers to communicate and thus arrange exchange of goods, services or resources. A free market is where such exchange occurs without interference from the government.

Information is a vital ingredient for any market. Both buyers and sellers need to have access to sufficient information to allow them to make rational decisions. So a market for health care must involve two groups: the buyers and the sellers, who interact to trade health care. Who would the buyers and sellers be in such a market? We all want good health and so most of us would be prepared, if necessary, to purchase medical treatment to cure an illness. This suggests that everybody is potentially a buyer (or consumer) of health care. More precisely, at any moment, a buyer would be any body who was ill or who wanted preventative medical treatment such as vaccination or who wanted guidance about their health. The sellers would be those people who could provide medical and health care services, such as doctors, nurses, physiotherapists, dentists and high street chemists.

The concept of supply and demand analysis helps to understand exactly how this spending of money operates in a market system. The answers to these and a thousand other questions can be found in the theory of supply and demand. This theory shows how consumer preferences determine consumer demand for commodities, while business costs underpin the supply of commodities. Finally, we will see how supply and demand are brought into balance by the movement of prices – by the prices.

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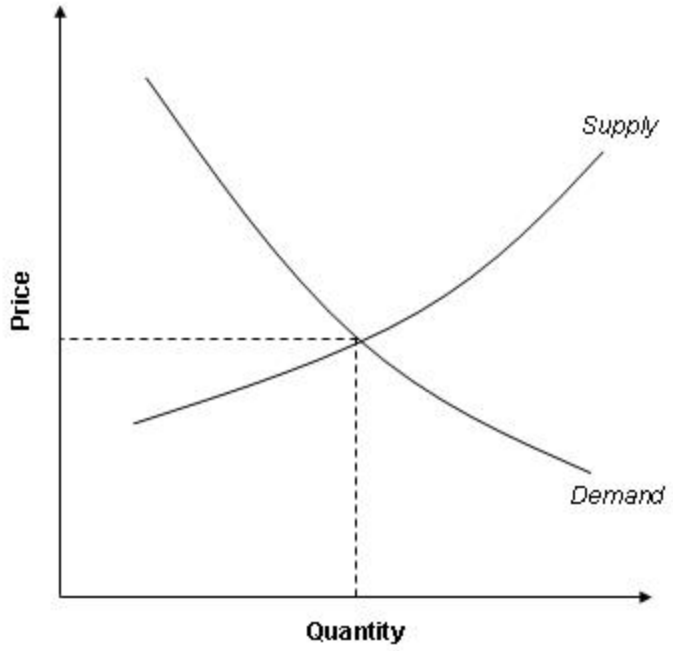

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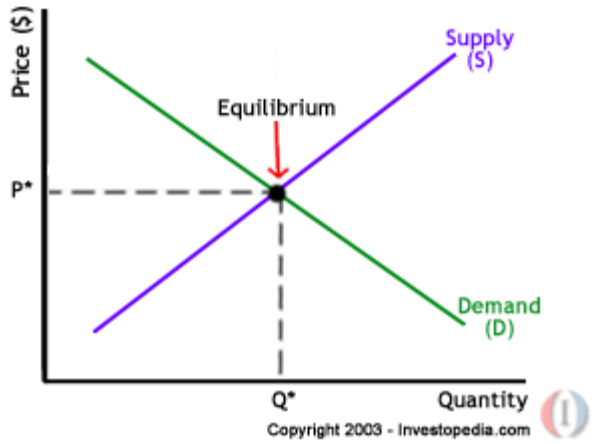
Equilibrium

When supply and demand are equal (i.e. when the supply function and demand function intersect) the economy is said to be at equilibrium. At this point, the allocation of goods is at its most efficient because the amount of goods being supplied is exactly the same as the amount of goods being demanded. Thus, everyone (individuals, firms, or countries) is satisfied with the current economic condition. At the given price, suppliers are selling all the goods that they have produced and consumers are getting all the goods that they are demanding.



Law of supply and demand
For higher prices less quantity is demanded and vice versa. Moreover, determinant of demand are taste, number of buyers, income, price of related goods and technology.

Law of supply
For higher prices greater quantity is supplied. Supply is always based upon a firm's cost.

Determinants of supply:

An important distinction between Need and Demand

Needs	Demands
• Someone's subjective idea (may be based on a formula applied objectively, but the choice to use the formula was someone's subjective idea). • Money is not a factor.	• Objectively observable as behavior in the market. • Money is a key factor. "Demand" is also called "effective demand," because it's expressed only by spending money

Healthcare financing:

Healthcare financing is much more than a matter of raising money for health. It is also a matter of who is asked to pay, when they pay and how the money raised is spent. This includes raising

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adequate funds for health, in ways that ensure people can use needed services, and are protected from financial catastrophe or impoverishment associated with having to pay for them. It provides incentives for providers and users to be efficient.

Sources of health care financing:

- ☐ general revenue or earmarked taxes
- ☐ social insurance contributions
- ☐ private insurance premiums
- ☐ community financing
- ☐ direct out of pocket payments

Each method

- ☐ distributes the financial burdens and benefits differently
- ☐ each method affects who will have access to health care financial protection

General revenue or earmarked taxes

The most traditional way of financing health care finances a major portion of the health care (especially in low income countries) Social insurance. It is compulsory. Everyone in the eligible group must enroll and pay a specific premium contribution in exchange for a set of benefits. Social insurance premiums and benefits are described in social compacts established through legislation. Premiums or benefits can be altered only through a formal political process.

Private insurance

Private contract offered by an insurer to exchange a set of benefits for a payment of a specified premium. Marketed either by nonprofit or for profit insurance companies, consumers voluntarily choose to purchase an insurance package that best matches their preference. Private insurance is offered on individual and group basis. Under individual insurance the premium is based on that individuals risk characteristics. Major concern in private insurance is buyer's adverse selection, under group insurance; the premium is calculated on a group basis. Risk is pooled across age, gender and health status.

Community based financing

Refers to schemes are based on three principles;-

- Community cooperation,
- Local self-reliance and
- Pre-payment

Factors for success of community financing are technical strength and institutional capacity of the local group.

Financial control as part of the broader strategy in local management and control of health care services;-

- Support received from outside organizations and individuals
- Links with other local organizations
- Diversity of funding
- Responding to other (non health) development needs of the community
- Ability to adapt to a changing environment

Direct out of pocket

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- Made by patients to private providers at the time a service is rendered
- User fees refer to fees the patients have to pay to public hospitals, clinics, and health posts not to private sector providers.
- Proponents of user fees believe that the fee can increase revenue to improve the quality of public health services and expand coverage
- Major objection raised against user fees had been on equity grounds

Community financing

- Technical strength and institutional capacity of the local group
- Financial control as part of the broader strategy in local management and control of health care services
- Support received from outside organizations and individuals
- Links with other local organizations
- Diversity of funding
- Responding to other (non health) development needs of the community
- Ability to adapt to a changing environment

Community based insurance schemes

- Primarily for informal sector
- Tends to cover all insured members of the community for all available services but have emphasis on primary health.
- Most financed from patient collections, government grant, donations, and such miscellaneous items as interest earnings or employment schemes
- Most NGOs have their own facilities or mobile clinics to provide health care.
- Total coverage is estimated to be about 30 million people (Ellis 1997).

Conclusion

- Role of health economists be recognized.
- Health financing cannot be dealt separately as it has got to do with good governance, economic growth, education.
- Social inclusion and financial protection seems to be provided through community based financing.

Who Funds The Health Sector In Kenya?

The health sector in Kenya obtains varying levels of funding from the traditional sources: public (government), private firms, households and donors. According to the 2005/06 National Health Accounts (NHA), households remain the largest contributors of health funds, at 35.9%, followed by the government, and then donors, who contribute approximately 30%.

Health Financing: How much is available?

The GOK funds25 the health sector through budgetary allocations to the MoH and the MoPHS and related government departments. As far as government spending is concerned, the Ministry of Finance sets three year budget ceilings for each sector in Kenya. In practical terms this means that the Ministry of Health creates a budget based on what the Ministry of Finance has said it will allocate for health expenditures rather than submitting a budget request based on actual needs. The Ministry of Health then disburses the funds it receives through its District Health Management Boards. The health budget shortfalls are manifest in the widespread lack of

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adequate drugs and pharmaceuticals, staff shortages and poor maintenance of equipment, transport, and facilities.

From the Kenyan People (Cost Sharing)

Over the past two decades, the GOK has pursued a policy of cost sharing to bridge the gap between actual budgets and the level of resources needed to fund public health sector activities. The revenue from the cost-sharing programme has continued to grow in absolute terms and as a percentage of the recurrent government budget. In 2002-03, cost-sharing contributed over 8% of the recurrent expenditure and about 21% of the non-wage recurrent budget of the MoH. However, because of the worsening poverty situation in the country, the MoH has changed its cost-sharing policy and replaced it with a "10/20" policy, in which dispensaries and health centres are not to charge user fees for curative care other than Ksh 10 or 20 for client cards28.

From the Donors

Kenya depends significantly on donor funds, many of which supplement the development component of the national health budget. In 2009/10, on-budget external resources accounted for 15.1% of all health spending in Kenya, or Kshs 7.1 million. In 2005/06, Kenya's health budget was made up to 11.3% by donor contributions. Over the years, Kenya's health sector has increased its dependency on external sources.

All donor commitments for Kenya's health sector—on and off budget—represent USD 99 million in 200831, down from USD 147 million in 2005. The United States is the largest bilateral donor, channelling funds through PEPFAR, the President's Malaria Initiative and USAID with USD 32.4 million in 2008. The United Kingdom also commits significant bilateral funds to the health sector (USD 30 million). When analyzing donors' actual health disbursements to Kenya', the picture looks different. In 2008, donors provided USD 116 million up from USD 102 million in the previous year. The biggest donors were GAVI, Global Fund to Fight Aids, Tuberculosis and Malaria and the United Kingdom. This graph shows that the US ranks only fifth.

Kenya National Budget Process

In Kenya the budgetary process comprises three main stages: drafting, legislation, and implementation and audit. Reforms aimed at improving the budgetary process have sought to impose greater fiscal discipline on the government among other objectives such as attaining allocative and operational/technical efficiency. The constitution requires the executive to raise and allocate resources for government projects within this rigorous budget process. The Budget cycle is based on the Medium Term Expenditure Framework (MTEF) cycle. The MTEF process prioritizes definition of a global resource envelope, determination of inter-sectoral allocations based on core functions and proposals of inter-sectoral allocations based on outcomes, activities and outputs. Public hearings and involvement of private and civil society in the budget preparation enhances transparency.

The MTEF process revolves around the activities of ten MTEF sectors, including the human resource development sector. To guide the preparation of MTEF, the Ministry of Finance issues to all ministries, departments and government agencies in August a circular detailing the guidelines with objectives for the financial year. The Circular-Budget preparation guidelines for budget 2010-11 for example directed that the preparation of the budget for the next MTEF period

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should be within the context of stimulating growth and creating jobs, reducing poverty and enhancing food security, and protecting the poor and most vulnerable of the society while improving government efficiency in delivering services.

The budget is prepared through Sector Working Groups (SWGs) comprising different ministries, departments, and agencies. The overall fiscal and macroeconomic framework—including strategies, outlook and forecast—determines the role of ministerial working groups (MWGs). The Ministry of Public Health and Sanitation (MoPHS) and the Ministry of Medical Services (MoMS) both appoint representatives to the ministerial working group who then work within the SWG. The two ministries fall under the human resource development sector.

The Sector Working Groups are urged to ensure that they only consider proposals for funding that support the achievement of the objectives and priorities outlined in the Medium Term Strategy of Vision 2030. The Circular from the Ministry of Finance encourages the involvement of districts and other stakeholders in the MTEF budget process through the sector hearings. The purpose of the Sector Hearings is to ensure transparent discussions of priorities of government for the next financial year with key stakeholders including development partners, civil society, private sector and research institutions.

Financial management is the process of achieving the organization's objectives in an efficient manner through planning (budgeting), acquiring, organizing, directing monitoring, controlling and reporting.

Why is it important?

1. The size of the health economy is large and growing
2. Role of government in the health care markets
3. Health care market is difference from other markets
4. Externalities

Financial Management in Health Care

Definition: Health financing
Health financing is making funding available to:

- Ensure choice and purchase of cost-effective interventions
- Give appropriate financial incentives to providers and
- Ensure that all individuals have access to effective health services.
  It requires equitable and efficient revenue generation and efficient purchasing of health inputs and services

Definition and Financial Management

Financial Management deals with the:

- Generation and allocation of resources to the most competing needs of an organization.
- Managing money in a health service is a complex and responsible work, which is done mainly by accountants or finance officers.

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- The health manager must have deep insight into financing issues and the transactions involved at different levels of the organization

Definition: Financial Management.

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- The health manager must have deep insight into financing issues and the transactions involved at different levels of the organization.

  Comprises three main Elements:
- Scope and contents of fiscal policies
- Guidelines and standards to ensure that funds are spent honestly and wisely to achieve publicly determined purposes
- Organizational structures and controls to effectively carry out fiscal duties and responsibilities

  CONSTITUTION OF KENYA Sec. 201 states;

❖ Principles and framework in Public Finance Sec. 215

❖ Revenue allocation commission has been created

PERFORMANCE IN THE PUBLIC SECTOR

❖ Efficiency

❖ Economy

❖ Effectiveness

TERMINOLOGIES AND DOCUMENTS INGOVERNMENTAL ACCOUNTS

❖ **Consolidated fund**

A fund where all revenues or other monies raised or received by the government are paid into

❖ **Appropriations in Aid**

Particular classes of revenue which the Treasury authorities allow an accounting officer to use in addition to the amounts to be issued from exchequer to meet expenditure

❖ **Paymaster General (PMG)**

The principal agent of the government and is considered as the banker for all government departments

❖ **Cashbook**

Shows all receipts and payments of cash

Common Documents

- Approved budget estimates
- Financial procedure manual
- Authority to Incur Expenditure
- Vouchers
- Requisition forms
- Local purchase orders / local service orders
- Cash book
- Cheques

Sources of Public and Private Sector Financing in Health

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There are two types of funds available to the health facility:

- Invisible money
- Visible money or cash

Invisible money:

- This money is not seen or handled.
- It is a paper credit given as allowance, allocation or warrant of funds

- **Visible money:**
  - This is the money that is seen and handled.
  - Visible money can be obtained through:
    - User fees
    - Fund raising
    - Donations
    - Pre-payment arrangements
    - Income-generating activities
    - Grants and other allocations under a cash budget

STEPS IN GOVERNMENT ACCOUNTING

1. Annual Estimates of both expenditure and revenue are prepared by the various ministries and submitted to treasury.
2. Presentation of the budget in June. Budget has estimates of government and expenditure for the year beginning 1st July to 30th June.
3. Committee of ways and means.
4. Spending by ministries.
5. Audit by Kenya National Audit Office
6. Public Accounts committee (PAC)
7. Supplementary estimates
8. Annual Estimates

- **Accounting Institutions and the nature of funds**
  The Treasury
- **Accounting officer**
  - Authority to Incur Expenditure (AIE) Holder
  - Entitlement and allowances
  - The Consolidated Fund
  - Consolidated Fund Services (CFS)
- **The Constitution, role of Parliament and Senate**
  Paymaster general

ROLE OF TREASURY

- Treasury advises all Accounting Officers on finance matters but the primary responsibility lies with respective accounting officers
- The Treasury appoints Accounting Officers in each Ministry, Department or Commission to be responsible for the money appropriated to it by Parliament.

ACCOUNTING OFFICERS

- Accounting officers are Permanent Secretaries or Heads of Departments, who are appointed in writing by the Permanent Secretary, Treasury.

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- An Accounting Officer's duties for which he is responsible to Treasury, include ensuring that:
  - ✓ No expenditure is made unless it is lawful, authorized, effective, efficient and economical;
  - ✓ Proper financial and accounting records are kept;
  - ✓ All contracts are complied with; All applicable procedures are followed in the acquisition and disposal of property and that adequate arrangements are made for the custody, safeguarding and maintenance of property;
  - ✓ Proper financial and accounting records are kept;
  - ✓ All contracts are complied with; All applicable procedures are followed in the acquisition and disposal of property and that adequate arrangements are made for the custody, safeguarding and maintenance of property;

AUTHORITY TO INCUR EXPENDITURE:

A controlling officer may, in accordance with the prevailing circumstances incur expenditure and authorize expenditure to be incurred against any provision shown in any subhead for which he is the controlling officer.

- No expenditure shall be incurred against any provision shown in any subhead except by or under the authority of the controlling officer.
- A controlling officer may, in respect of any subhead for which he is the controlling officer, by allocation warrant signed by him or by a public officer authorized generally or specially by him, authorize any other controlling officer to incur expenditure or authorize expenditure to be incurred against any provision shown in that subhead;
- For any purpose within the ambit of that subhead
- In accordance with the terms of the allocation warrant.
- Dealing with Imprests
- It is issued for specific purpose and any payment from it must be only for the purpose specified.

Requirements before issuance of imprest

- Adequacy of funds in the vote
- Objective of the journey cannot be achieved by any other means
- Applicant has no outstanding imprest
- Certification by the Accountant that the applicant has no outstanding imprest and that newly applied amount has been recorded.
- Certification by the vote book controller that funds are available.
- Temporary imprest holder should surrender the imprest within 48 hours after return to the duty station.
  - Failure to which the Accounting Officer should recover the full amount outstanding from the Officers salary.
  - Recovery of the imprest by installment must only be authorized by the Accounting Officer and be accompanied by appropriate disciplinary action against the officer.

FINANCIAL REGULATIONS AND PROCEDURES

What are they?

- These are firm and well defined rules, guidelines and procedures that provide both the institutional and the legal framework for managing and accounting for public funds.

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It takes two forms namely:

- Establishment of Institutional Framework and
- Establishment of the legal Framework (Procedures) for dealing with financial affairs of the country

Financial Management Policies

The responsibility for management, supervision, control and direction of all matters related to the financial affairs of Kenya are vested in the Minister of Finance (Treasury), who discharges his responsibilities through formulation and promulgation of regulations under powers given to him in the Exchequer and Audit Act, (Cap.412).

PRINCIPAL SOURCES OF FINANCIAL REGULATION

Government Financial Management Act (GFMA)

What is a budget? The budget is more than just a number of figures

The budget should express: targets of government policy measures and instruments to reach these targets costs connected to the targets and measures

Three elementary questions in the budget

- What is it that you want to achieve (what's your goal)?
- What measures are you going to take to achieve that goal?
- What is it going to cost?

Budgeting in the public sector :

- Political instrument to allocate scarce resources to priorities
- Managerial tool to explain how programs achieve policy objectives
- Economic instrument to influence the economic growth of the country
- Control tool to hold managers accountable for outcomes & outputs.

Different budget formats

(1) Line Item Budgets

- The expenditure are expressed in considerable details but activities being undertaken are given little attention
- It shows the nature of the spending but not the purpose
- The amount in this type of budget are frequently established on the bases of historical costs that have been adjusted for anticipated changes in cost and activity levels
- Control oriented – it is a form of financial control rather than effectiveness
- Shows exactly how much is spent on each item of expenditure
- Focuses on 'inputs' not 'outputs' or 'delivery'
- Itemizes payments (and receipts) by standard categories
- Helps to ensure aggregate fiscal discipline

Advantages and Disadvantages of Line Item Budgets

Advantages:

- Are simple to draw up
- Provide an analysis of expenditure on inputs
- Allow for easy control over inputs used
- Easy for politicians to see an amount spent per budget item

Disadvantages

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- Provides little information about what the budget is spent on
- Gives no indication of how funds are used within programmes
- Does not show the differences in expenditures on managerial, administration and operational staff

Focuses on control of funds instead of achieving results.

2 Programme Budgeting

1. process that relates resources to proposed and actual results
2. Clusters of related activities that represent the highest level of classification of the work undertaken by a department in carrying out assigned responsibilities
3. Allocates money to functions/activities and focuses on total cost of providing a service, supported by line item detail.
4. Stresses on the end product, not the means

Key Concepts for programme based Budget (PBB)

- Vision :A realistic and attractive future scenario for the organization
- Mission: A broad definition of why the agency exists, what are the main activities and who are the targeted beneficiaries
- Programme:
  - A group of independent but closely related services and activities which contribute to the programme goal.
  - A grouping of activities that contribute to a common strategic objective.
  - Programmes are divided into component sub-programmes

Objectives of Programme Based Budget

- Making public management results-oriented
- Improving efficiency/effectiveness of activities;
- Using performance information for setting targets and priorities;
- Guiding resource re-allocation with greater information;
- Making efficiency savings wherever necessary.

Line Item Budgeting vs. PBB

Line Item Budgeting	**PBB**

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► Focuses on compliance/use of inputs ► Presentation in terms of categories of inputs (line item) ► No information on performance ► No information on unit costs of goods and services provided; ► Lack of information on future funding needs. ► No clear distinction between externally "administered" items (e.g., level of grants or subsidies) and "controlled" items (subject to managerial control)	► Focuses on outputs/results ► Presentation by activities/tasks performed (fewer appropriation items) ► Performance measures defined ► Costing facilitated by defining outputs; ► Greater predictability in funding needs ► "Controlled" and "administered" items are identified and separately recorded

Why Move to a PBB System?

1) To improve efficiency and cost-effectiveness in public financial management using techniques and planning frameworks based on international best practice;
2) To improve resource allocation by identification, ranking and prioritization of programmes ;
3) To improve the efficiency of budget management by including performance information in budget documents to assist monitoring and evaluation;
4) To use performance information to guide outlays on different activities, providing a more structured framework for budget resource allocation, both within and across sectors.

Challenges of Programme Based Budgets

- Time consuming
- Requires a lot of information
- Requires a major change in the way a Ministry operates and change can be difficult
- Requires a paradigm shift in management behaviour

Financial Recording and Reporting Documents

- General voucher
- General allowance claim form
- Imprest request/retirement voucher
- Cash reconciliation sheet:
- Monthly schedule of outstanding imprests
- General ledger card
- Cash book
- Quarterly budget expenditure report (QBER)
- Final financial report
- Income register

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- The Budget is an estimate of Government expenditure and revenue presented to parliament by MoF before June 20th each year.
- Seeks approval for expenditures for the financial year that starts on 1st July for the vote of all Ministries.

Budget classification

1. Recurrent Estimates(votes) for:

a. Services
b. Salaries

2. Development estimates (vote) for all development projects/programs/activities in all the ministries

Types of Budgets

Medium-Term Expenditure Framework (MTEF): This is a three-year rolling budget process aimed at aligning policy, planning and budgeting.

Annual budget: An annual estimate of Government expenditures and revenues for all ministries.

The Revised/Supplementary Budget

- Prepared in December of every year.
- Enable ministries and departments adjust their budgetary allocation in line with the expenditure trends observed over the past six months.
- Usually treasury only allows reallocations where savings can be realized from other expenditure areas.

The budget preparation process

Sector Working Groups (SWGs)

- The MTEF process grouped ministries in economic divisions referred to as sectors.
- Various ministries in a sector submit their budget proposals to the sector for funding (popularly known as bidding process).
- The requests from the various ministries are evaluated by the Sector Working Groups (SWGs) which comprises officers from the respective ministries in the sector, Treasury and Ministry of Planning officials.

Confirm this conforms to new Constitution requirement

1. Ministerial Public Expenditure Review
2. Sector Working Groups
3. Sector Hearing Stage
4. Annual Budget Preparation
5. Budget Statements

Budget implementation

- From 1st July (which year 2013 Ministries operate on Vote on Account whereby they are allowed to spend only one half of their provisions/ allocations until the Appropriation bill is passed
- Field stations are given Authority to Incur Expenditures (AIES) and cheques / checks qu Sources of Government Finance/ revenue
- GOV. Finance = All monies receivable by the Government for financing its services and implementation of development programes
- Classified into two:
  1. Recurrent Revenue

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- Finances needed to pay for the operating costs of running the government services.

Main sources of this revenue are:

- ❖ Fines and fees
- ❖ Taxes e.g. VAT, P.A.Y.E
- ❖ Duties e.g. custom duty, excise duty
- ❖ Investment revenue etc.

1.Appropriation in Aid

- Is a form of revenue that the Treasury allows an accounting officer to collect and use in addition to amounts issued from the exchequer.

2. Development Revenue

Finances needed to implement development programes and projects.

Main sources are:

- Sale of non strategic assets
- Grants
- Loans quarterly in order to implement their planned activities.

Budgeting-Facility level

- ❑ The budgetary process is carried out in tandem with the planning process.
- ❑ Technical and financial units work together in deriving costs of what is being proposed in the plan.

Budgeting Process:

- Consider each objective separately.
- Examine the approach accepted by the planning group noting the measures being proposed.
- Define the operational targets and specify the operational output and the quantification.
- List the activities, including support activities necessary to reach the operational targets, and quantify these activities Assess the costs related to activities.
- Develop a table that shows for each stratum of the plan, the objectives, approaches (strategies), activities, targets and costs.
- Apply the costs of each target to a line-item budget format by categories (personnel, supplies, equipment, transportation, emergency funds for, say, malaria epidemic control and research).
- Prepare a clear presentation of the budget linked to the plan and be ready to defend it. Determine whether existing budgetary control procedures must undergo some changes and formulate recommendations for the proposed changes.
- Note the major budgetary implications of a decentralized organization.
- Costs of large categories of activities may already be known from previous planning exercises or from a continuous programme of activity monitoring. If not, analyse tasks for major activity linked to every target and determine:

  The time taken to accomplish every task.

  The labour costs to estimate the staff needed.

  The work that can be achieved in one day, one week, and one month.

  The quantity and type of supplies and equipment required to accomplish the tasks and the costs.

- ▪ Factors that may assist and should be taken into account in assessing cost/efficiency are:
- ▪ Human resources relative to plans, and efficiency of human resources in relation to expectations.

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- Financial resources intended for implementation and correct use of these resources.
- Suitability of the sequence of activities and the appropriateness and timeliness of the logistical support.
- Collaboration with other services in the health, social and economic sectors.

Applicability of operational plans.

- Work schedules.
- The methods applied.
- During the planning phase. the cost analysis will be stratified so as to permit analysis of each activity as well as of the programme as a whole. This operation may require additional human and other resources and may considerably increase the volume of reporting, but the results will permit substantial improvements in the programme's operating efficiency and considerable savings in the long term.

Cash management

- ✓ Cash is the most important asset an organization can have. Its availability or lack of it can determine the success or failure of an organization.

Why manage cash?

- ✓ Inflows and outflows do not coincide i.e. cash needs vary.
- ✓ Cash is usually managed through preparation of a cash budget that can be prepared to capture the inflows and outflows on monthly basis for the next year and quarterly for second year.

  Areas to monitor
- ✓ Imprests - especially for projects that require liquidation of advance before additional funds can be released.
- ✓ IOU's – they are illegal
- ✓ Substantial cash purchases – room for rent seeking
- ✓ The 'big three' i.e. Receivables, Payables and Inventory

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UNIT FIVE: HUMAN RESOURCE MANAGEMENT
OVERVIEW

- Key concepts in human resources for health
- Human Resource health action framework
- Health policies and laws at global, national, county and institutional levels
- Human Resource for Health functional areas
    - Human Resource Planning
    - Recruitment and Selection
    - Training and Development
    - Performance management
    - Compensation and Rewards
    - Employee Wellness
    - Employee and labour relations

Definitions of concepts

Human resource management (HRM) is the systematic acquisition, maintenance, utilisation and outplacement of work force to achieve organisational objectives. It has several functional areas like planning, training and development, performance management amongst others.

Human resources for health

Human resources for health management is the integrated use of policies, systems and management and leadership practices to plan for necessary staff and to recruit, motivate, develop, and maintain employees so that a health institution or organization can meet its goal
Source: USAID MSH e-handbook for Leaders and Managers (2010)

Components of HRM

HR is the systematic acquisition, maintenance, utilization and outplacement of work force to achieve organizational objectives.
Consists of several functional areas:

- Human Resource Planning
- Recruitment and selection
- Training and development
- Performance management
- Compensation and rewardsHR is the systematic acquisition, maintenance, utilization and outplacement of work force to achieve organizational objectives.
- Consists of several functional areas:
- Human Resource Planning
- Recruitment and selection
- Training and development
- Performance management
- Compensation and rewards
- Employee wellness
- Employee relations

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- Retention and motivation
- Exit management

Human Resource Planning

The process through which an organization ensures determines current and future requirements of:

- the proper number of employees
- appropriate skills
- in the right jobs
- at the right time to achieve organization's objectives.

It involves:

- **Job analysis** refers to determining skills, duties, knowledge, tools, environment and the person required for job performance in the organization
- **Staffing needs analysis-** systematic reviewing human resource requirements to ensure that the required numbers of employees with the required skills and resources are available when needed
- Human resource planning compares the present state of the organization with its goals for the future
- Then identifies what changes it must make in its human resources to meet those goals

Recruitment and selection

- **Recruitment** is the process of searching the candidates for employment and stimulating them to apply for the jobs in the organization.
- A process of finding and attracting capable applicants for employment.
- It is the process of discovering sources of manpower to meet the requirement of staffing schedule and to employ effective measures for attracting the manpower in adequate numbers to facilitate effective selection of an effective workforce.
- Searching for prospective employees
- **Selection** refers to choosing the most suitable candidates from those who apply for the job. It refers to offering jobs to desired candidates
- A process of differentiating between applicants in order to identify and hire those with a greater likelihood of success in a job
- A long process which begins with interview of applicants and ends with employment contract.
- In practice selection differs from one organization to another and between different jobs.

Training and Development

Deals with:

- Orientation and induction
- Training & development
- Career planning & Pathing
- Career Counseling.

Orientation and Induction

- A procedure for providing new employees with basic background information about the firm such as rules

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- New employee orientation effectively integrates the new employee into your organization and assists with retention, motivation, job satisfaction, and quickly enabling each individual to become contributing members of the work team.

Training and Development

- **Training**- refers to a learning experience organized by an employer to develop and sustain behaviors that bring about improvement and personal growth. It is organized for a specific period of time but outcomes can take long periods to be realized.
- Training of employees is planned learning experience which provides staff with knowledge, attitudes and skills to enable them perform a specific job either today or in the future

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- **Development**- learning that goes beyond today's current job needs, hence this includes training and education but goes beyond to target learning experiences which assist employees in personal life and growth
- It prepares employees for future work, increase capacities to perform in the current jobs. e.g. preparing them for future challenges
- **Career development-** formal approach used by organization to ensure that people with proper qualification and experiences are available when needed

- **Career planning-** an ongoing process whereby an individual sets career goal and identify the means to achieve them
- **Career pathing-** identifying career goals and means to achieve them

Process of Training and Development

The process takes the following steps:

i. **Training Needs Assessment**

- Identifying gaps in knowledge and skills of workers which inhibit the performance.
- It aims at identifying critical skills gaps, develop a sound base for management decisions on training, realise demand driven training, improve overall performance and service delivery

Training Projection

- These are plans based on identified training needs
- They assist in preparation of projections that guide in nominations of officers for training
- Aimed at developing HRD plans based on needs identified through the training needs assessment
- It assists in earmarking those to be trained

Training Budget

- Financial implications of each course is taken into consideration depending on availability of funds

Career Pathing

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- Career path is a succession of different jobs through which an employee may move within an organisation. People move through their careers either upwards through promotion or by enriching their roles to utilise more skills
- Career expectations of employees may differ significantly and therefore career paths should identify realistic options available within an organisation Career pathing
- help employees identify skills required
- align and integrate personal aspirations with organisational objectives
- identify career paths that points in all directions, not only upwards
- provide employees with the opportunity to develop themselves and their career
- provide mutual benefits for both the organisation and individual

Succession planning

- The planning takes into account the fact that employees will not remain indefinitely in the service of the organisation and provides a plan and process to deal with change that occur when employees leave
- The emphasis is on employee development, so that the organisation has a pool of qualified candidates who are willing to participate and contest for key positions or positions in key areas , if they become vacantOrganisations should:
- assess their present and future needs based on their plans, strategic goals and objectives, or their priority programmes and projects
- use the capabilities of existing employees to meet those needs
- develop a plan to fill the vacancies of those in authority who leave or are promoted

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- assess their present and future needs based on their plans, strategic goals and objectives, or their priority programmes and projects
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Steps to consider in succession planning

Step 1 Identify key areas and key positions
Step 2 Identify capabilities for key areas and key positions
Step 3 Identify interested employees and assess them against capabilities
Step 4 Develop and implement succession and knowledge transfer plans
Step 5 Evaluate effectiveness

Benefits of good succession planning

- Increases the level of employee engagement through career planning, a challenging work, training and development, career advancement and development horizontal
- Improves the capacity of the organisation to achieve its business objectives through pools of qualified candidates for positions and key sectors
- Allows managers to better appreciate the capabilities and skills, experience, diversity, language skills and interest in learning of their employees
- Prevents the loss of institutional memory when employees leave the organisation

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- Allows for cost savings through the creation of a more committed and productive workforce
- Creates a more efficient and effective public service in the long run

Challenges of good succession planning…

- Lack of discernment in the succession plan, we rely on employees who are not interested or motivated, and who have not qualified
- A lack of training and development will ensure that an employee will not be ready to get a promotion
- A plan that does not promote employees in a timely manner
- Poor communication of the succession plan could lead to confusion and turmoil within the organisation and staff

PERFORMANCE MANAGEMENT

- Performance management is a means of getting better results from the organisation, teams and individuals by understanding and managing performance within an agreed framework of planned goals, standards and competence requirements
- It is a process of establishing shared understanding about what is to be achieved and an approach to managing and developing people in a way that increases probability that goals will be achieved in the long and shorter term and in the most efficient way
- A process that consolidates goal setting, performance appraisal, and development into a single, common system, the aim of which is to ensure that the employee's performance is supporting the company's strategic aims

Steps in performance management

Role definition

- Agreeing on key result areas and competency requirements (Job descriptions)

Performance agreement (contract) or work plan

- Objectives
- Mechanism for measuring performance
- Competencies needed to deliver required results

Personal development plan

- Intended actions for competency improvement for performance
- Managing performance throughout the year
- Taking action to implement performance agreement and personal development plan (continuous feedback, informal progress reviews, updating objectives and counseling and remedial measures)
- Performance review or appraisal
- Formal evaluation stage (achievement, progress and problem/challenges)
- If need be revision of performance agreements and personal development plans

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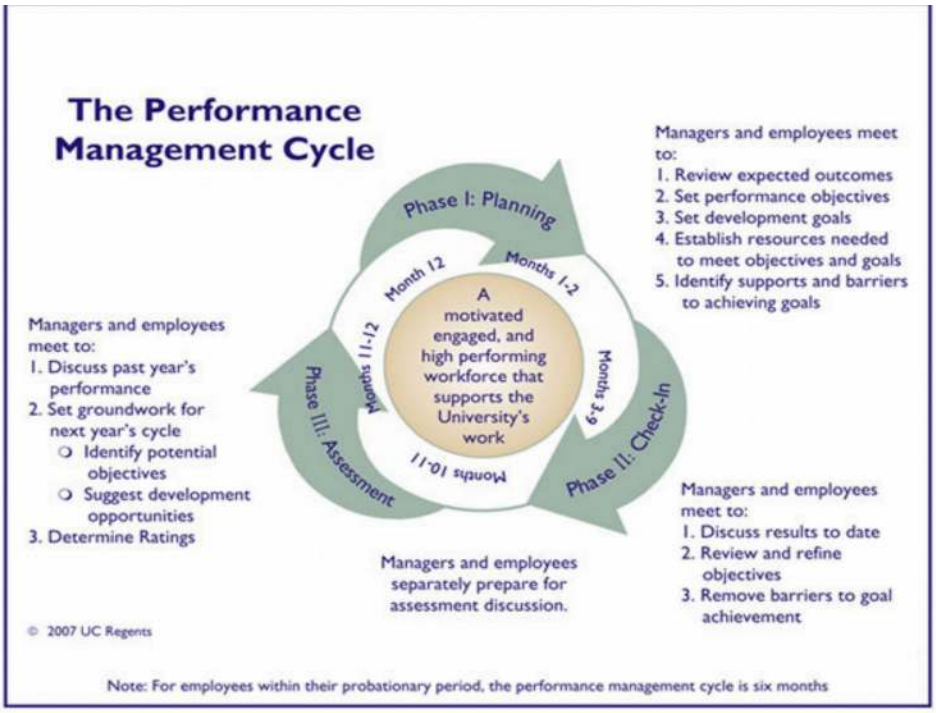

Performance appraisal

- This refers to evaluating an employee's current and /or past performance relative to his or her performance standards

Performance Appraisal tools

- Formal annual performance appraisals form
- Probationary reviews
- Informal one-to-one review discussions
- Skill- or job-related tests
- Survey of opinion of others who have dealings with the individual
- 360 degrees assessment
- Balance score card (teams)
- Graphic rating scale
- Observation on the job

What to Appraise

❖ Assessing overall job performance
❖ Factors to evaluate:
  - ➢ Technical competencies
  - ➢ Organizational Values
  - ➢ Efficiency of accomplishing work tasks
  - ➢ Observance of safety rules
  - ➢ Timeliness
  - ➢ Communication skills
  - ➢ Customer service

Performance Appraisal process

- Deciding which targets to appraise

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- Collecting the information needed to measure performance
- Comparing results with relevant targets
- Judging the degree to which targets were met
- Deciding on the course of action(rewards and sanctions)

Challenges of conducting Performance Appraisal

- Unclear standards because of ambiguous traits and degrees of merit
- Halo effect-the influence of the rater's general impression
- Central tendency-rating all employees average
- Leniency or strictness of supervisors
- Bias – the tendency to allow individual differences such as personality ,race, age ,sex to affect ratings

Effective performance appraisals

- Effective performance appraisals ensure that:
- employees and supervisors learn and understand problems associated with the process
- The right tools are used
- Supervisors are trained to avoid errors of central tendency and leniency
- Outside influences are controlled
- Employee diaries are kept for objectivity and accuracy
- The use of any of these methods depends on the purpose of the appraisal

Compensation & Rewards

Compensation –Covers all rewards that individuals receive as a result of their employment and consequent expenditure of their efforts, time and energy in the organization.

It consists of:

a) Pay/base/basic pay – the money that a person receives for performing a job e.g. salary/wages.
b) Compensation –Covers all rewards that individuals receive as a result of their employment and consequent expenditure of their efforts, time and energy in the organization. It consists of:
c) a) Pay/base/basic pay – the money that a person receives for performing a job e.g. salary/wages.
d)) Incentives- compensation given to employees for extra tasks, better performance or unique contribution, e.g. over time pay, commissions.
e) -These will inspire the staff to take up extra responsibilities, improve performance and have job satisfaction.

Role of compensation and rewards

An effective workforce strategy will address the core challenges of:

- improving recruitment
- improving the performance of the existing workforce
- Slowing the rate at which workers leave the health workforce

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Incentives can play a role in all these areas by providing a means through which health systems can attract and retain essential and highly sought-after health care professionals

Employee Wellness

It encompasses health and safety.

Safety – involves protecting employees from injuries caused by work related accidents or activities

Health – employees freedom from illness; mental, physical , spiritual and their general well-being.

These can be achieved by providing medical cover, protective equipments, safety and health training. A state of complete physical and social wellbeing and not just the mere absence of disease.

It has various dimensions i.e. Social, Occupational, Spiritual, Physical, Intellectual, Emotional, Environmental Financial Mental Medical and Vocational

A healthy work environment is one in which there is not only absence of harmful conditions but an abundance of health promoting condition

Wellness can be assured by ensuring:

- Continuous assessment of risks to health
- Appropriate provision of information and training on health issues
- Availability of health promoting organisational support practices
- Effective management of occupational health and safety risks Personal security
- Personal security

Introduction labour laws

- Labour or employment laws are designed to protect both workers and employers
- Labour laws are the body of laws, administrative rulings, and precedents which address the legal rights of workers and their organisations
  - The laws address issues of equal pay for equal work, discrimination, benefits e.g. insurances, pension, and others (job security)
- Good labour laws are as good as their enforcement. Legislation is useless without law enforcement to back it up

Some Labour laws

There are International and national labour laws. For example in Kenya, the following are some of the labour laws

- The Employment Act, Cap 226
- The Regulation of Wages and Conditions of Employment Act, Cap 229
- The Trade Unions Act, Cap 233
- The Trade Disputes Act, Cap 234
- The Factories and Other Places of Work Act, Cap 514

Employee and Labour Relations

Employee relations covers all types of interactions among people at the work place- Its their conflict, cooperative efforts and group relationships.

This must be cordial to boost the morale of the workers and consequently their productivity. It also covers relations among employees themselves.

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- Management hold the key to creating a conducive environment for such interaction to take root and flourish.
- Labour Relations- recognition of union membership and the willingness by the management to resolve disputes through the union.
- Employees should be given the freedom to join trade union and to participate actively and as stipulated by the Trade Unions Act and ILO.

overview

- Rationale and principles of procurement
- Procurement regulations and ethical guidelines for good procurement practices
- Roles and responsibilities of various bodies and actors in the procurement procedure
- Procurement procedures and variations at each stage based on the type of goods and services
- Procedures for contracting
- Supply Chain management tools for inventory, management, disposal of stores and equipment
- Investment plan application – designing a procurement plan for goods and services
- Challenges to supply chain management in health systems

Current classification of procurement entities

Class A are Ministries and State Corporations

Class B are City Councils, Universities, Judiciary, Commissions, Colleges, Cooperative Societies, Parliament, Districts, Provincial Hospitals and SAGAs

Class C are Municipalities, County Councils, Urban Councils and Schools, District Hospitals, Sub District and Dispensaries, CDF

Anatomy of Kenya's Public Procurement and Disposal Act

The purpose of the Act

- To establish procedures to achieve the following objectives:
- To maximize the economy and efficiency
- To promote competition and ensure that the competitors are treated fairly
- To promote the integrity and fairness of those procedures
- To increase transparency and accountability in those procedures
- To increase public confidence in those procedures
- To facilitate the promotion of the of local industry and economic development

Application of the Regulations.

These Regulations apply to public entities as enlisted under Section 3(1) of the ACT that is-

- Any body using public assets in any form of contractual undertaking.
- Companies owned by public entity
- Any body in which the Government has a controlling share

Acquisition of goods and services not classified as procurement according to the Act (Section 4(2)

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- Retaining of services of an individual for a period of term if the individual works as though one was an employee.
- Acquiring stores and equipment disposed by a public entity.
- Acquiring of services provided by Government
  or a department of the Government.

Conflict with other Acts

- This act prevails on matters of pro
  In relation to donor funds Conflict with international agreements
- The Act prevails except in instances of negotiated grants and loans.
  Conflict with Donor conditions
- Donor conditions to prevail with respect to procurement

BODIES AND ACTORS INVOLVED IN PROCUREMENT PROCESS: ROLES AND FUNCTIONS

Key Bodies and Actors: Membership, roles and functions

- Public Procurement Oversight Authority (PPOA)
- Public Procurement Oversight Authority and the Director General
- Public Procurement Oversight Advisory Board (PPOAB)
- Public Procurement Administrative Review Board
- Internal Organization of Public Entities
  - Procurement Unit
  - Procurement committee
  - Tendering committee
  - Roles of Heads of Departments
  - Evaluation committee
  - Inspection and acceptance committee
  - Disposal committee

There ought to be separation of roles to ensure that procurement contracting process is accountable .

Within a Public Entity the procurement contracting functions should hence be separated. Why?:

- To avoid conflicts of interest.
- To promote the integrity and fairness of the procedures {(Sub-Section 2(c) of the Act}
- To increase transparency and accountability in the procedures {(Sub-Section 2 (d) of the Act}

Hence- A Public Entity shall establish a tender committee , procurement unit and other bodies required under the regulations for making procurement decisions {(Subsection26 (4)}

Bodies involved in Regulation of Public Procurement

Public Procurement Oversight Authority (PPOA) – Functions

- Monitoring the performance of the public procurement system and report to the Minister (M&E)
- Assisting in the implementation of an efficient and effective public procurement system
- Preparation of manuals
- Providing advice and assistance to procuring entities
- Issuing directions to procuring entities

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- Offering other functions provided under this Act.

Promotion of Professionalism

- The Authority to assist in the establishment of an examination body and professional association for procurement professionals.
- Training and professional development
- Ensuring engagement of procurement professionals

The Director General

- The Chief Executive of the Authority.
- Appointed by the Advisory Board
- Term of office – 5 years, renewable once.
- Conditions for termination by the Advisory Boards:
  - Incompetence
  - Infirmity
  - Conviction
  - Corruption
  - Employed in other public office
  - Bankruptcy.
  - Sources of Funds

- **Funds of the authority**
  - Money appropriated by Parliament
  - Loans/grants
  - Revenue/fees for services rendered
  - Capacity building levy

Public Procurement Oversight Advisory Board (PPOAB)

Composition – 9 members appointed by the Minister from persons nominated by prescribed (private and public) organizations. (First schedule for prescribed organizations), and the Director-General.

Functions:

- Approve estimates of revenue and expenditures of the Authority.
- Advise the Authority
- Recommend appointment or termination of Director- General in accordance with this Act.
- Other functions and duties provided within the Act

Public Procurement Administrative Review Board

Continued from the previous Public Procurement Regulations. -Legal Notice No 51 of March 2001

Composition of the Board shall be as prescribed in the regulations.

The Authority shall provide administrative services to the Review Board.

The Internal Organization of Public Entities Relating to Procurement

Significance of the Internal organization

Decision making process

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- **in a systematic and structured manner.**

Decision making structure

- Oversight function by the Accounting officer/CEO at PE level
- Establishment of Tender Committee, Procurement Committees and Procurement Unit
- Head of Departments participation in the procurement process.
- Coordination and Secretariat services offered by the Procurement Unit
- Establishing and staffing Procurement Unit with procurement professionals.

Planning.

- Planning Procurement annually within approved budgets

Compliance Responsibilities

- Accounting officers to ensure that the Act is complied with

Outsourcing of procurement by the PE or by the Authority

- To a Procuring agent who shall comply with the Act, Regulations and the Authority's circulars
- To another Procuring Entity
- ***NB**: Authority to register procuring agents*

THE TENDER COMMITTE

The Roles- Sub-section10 (2) of the Regulations

- **To review, verify and ascertain that all procurement and disposal has been undertaken in accordance with the Act, Regulations and terms set out in the tender document.**
- **To approve, select, award contracts to successful bidders where value exceeds threshold in 1st Schedule**
- **To ensure availability of funds for procurement under consideration**
- **To ensure intended payment not to exceed prevailing market prices**
- ***Pre- adjudication action: Procurement Unit to carry out periodic market surveys {(Reg.8 (3) (z)}.***

The Tender Committee: ***roles***

- *Review selection of procurement method (most preferred-open tender)*
- *Ensure any other procurement method is in line with the Act and Regulations.*

  Pre- adjudication action:
  - *Selected procurement method is to be included in the Procurement Plan (Reg.21 (1) (h) and*
  - *Complying with conditions for use of "Alternative Procurement Procedures " Part VI of the Act.*
- *Review and approve aggregations of procurement, packaging or use lots if proposed.*

  Approve list of tenderers in case of restricted tendering pursuant to Reg.54 (3)
  - *Invitation of tenders from at least 10 persons selected Sub-Reg.54 (3)(a)*

  Fulfilling the conditions for use of Restricted Tendering Section 73 (2) of the Act.

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Approve list of persons qualified to submit proposal pursuant to **Section 80 of the Act**

Pre- adjudication action: *Examination and evaluation of Expressions of Interest to determine qualification for invitation to submit proposal*

Approve negotiation under **Section 75 and 80 of the Act**

Pre- adjudication action:

- *Negotiation is allowed in respect of Direct Procurement (Section 75 of the Act) and*
- *In respect of Selection of the most advantageous proposal determined with regards to Section 80 of the Act.*
- ***NB:*** *Requirements for negotiation should be stipulated in Bid document or of Letter of Invitation as appropriate.*
- Approve: Amendments of contracts previously awarded by the Tender Committee in accordance with **terms and conditions in the Regulations**
- **Approve list of persons to be given Request for Quotation pursuant to Reg.59 (3)**
- ***Pre- adjudication action:***
- ***Selection of qualified persons from maintained list and/or***
- ***From knowledge of the market .***
- ***NB: Selected list of qualified persons can be supplemented by other known sources in the market***
- **To review the quarterly reports on quotations that have been awarded by procurement committee.**
- ***NB: This is in accordance with Reg 10 (2) (a) which states that Tender Committee shall review, verify that ALL procurements and disposals have been undertaken in accordance with the Act and the Regulations***
- **To undertake any other functions and duties as are provided under the Regulations or as may be stipulated by the Authority.**

The Tender Committee: Composition

Established in the manner set out in Second Schedule to the Regulations to consist of:

- Not less than FIVE members
- Have as its secretary, the procurement professional in charge of procurement unit

Categories of Tender committees in Second Schedule cover Procuring entities in

The Tender Committee: Procedures for meetings

line with Section 3 of the Act- *Definition of Procuring Entities*

Tender committees membership to include ***alternate members***

Only alternate member to attend meetings whenever the member is unable to attend

Quorum to be FIVE including the Chairman

Decision of the Committee shall be by ***consensus***

In case of no consensus decision shall b Members may be ***paid honoraria***

The committee shall cause to be prepared the ***minutes*** *of the meetings*

The minutes shall include:

- *A register of attendance*

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- *Date of the meeting*
- *List of all matters considered and decision made, reasons for rejections, clarifications, minor amendments*
- *A notes regarding evaluation*
- *Conflicts of interest declared and* ***any dissenting opinion among members***
- *Such other records as may be necessary.*

e made through ***voting by simple majority***
Where there is a tie during voting ,the Chairman shall have a ***second and a casting vote***
A member with interest (direct or indirect) in the matter being deliberated upon shall ***declare the interest*** *and shall not participate in the deliberation*

Procedures for meetings

Members may be ***paid honoraria***
The committee shall cause to be prepared the ***minutes*** *of the meetings*
The minutes shall include:

- *A register of attendance*
- *Date of the meeting*
- *List of all matters considered and decision made, reasons for rejections, clarifications, minor amendments*
- *A notes regarding evaluation*
- *Conflicts of interest declared and* ***any dissenting opinion among members***
- *Such other records as may be necessary.*
- ***Committee may also invite two more observers for contract with estimated value above Kshs 50 Million***
- ***At leased one observer to come from a recognized private sector organization or discipline relevant to the procurement under consideration***
- ***Failure of an observer to a meeting shall not nullify the procurement proceedings .***

The Tender Committee

Adjudication of evaluated bids to ensure:

- *Compliance with product specifications*
- *Delivery /Lead-Time.*
- *Supplier's economic standing*
- *Supplier's legal standing-whether qualified to operate as supplier/vendor of the proposed goods services or works or not.*

Adjudication of evaluated bids to ensure:

- *Supplier's relevant experience*
- *Supplier's technical capability to perform the proposed contract*
- *Supplier's responsiveness to the tender technical requirements*
- *Price for delivering the goods or rendering the services*
- *Total cost*

Procurement Committee

Roles

- *Responsible for procurement below the threshold for Tender Committee*
- *May approve or reject submissions with reasons.*
- *May approve submission subject to* ***minor*** *clarification by procurement unit*
- *Shall not modify any submission or reject submission without justified reasons*

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- *Any rejected submission maybe resubmitted (with further clarity or details)*
- *Committee to provide explanations and justifications of its decisions.*

Composition

- *An official delegated the role of being a Chairman by the Head of the Procuring Entity*
- *The Finance officer or an officer carrying out related functions*
- *Three members appointed by Accounting Officer*
- *Secretary-an officer appointed by Head of Procurement Unit.*

Procedures for Meetings

Quorum*-Chairman and at least two other members*

A member unable to attend a meeting may delegate authority to a appropriate official

The committee to ensure that the delegated authority is to an official with ***appropriate skill and experience***

Decisions of the committee shall be by consensus, otherwise the decision shall be through ***voting*** *by simple majority*

In case a of a tie in voting the Chairman's vote shall have as second or casting vote.

- *A member with interest (direct or indirect) in the matter being deliberated upon shall declare the interest and shall not participate in the deliberations.*
- *The Committee may invite independent observer or member of procurement unit to explain submission or provide technical advices.*

The Procurement Committee to adjudicate evaluated bids to ensure:

- *Compliance with product specifications*
- *Delivery /Lead-Time.*
- *Supplier's economic standing*
- *Supplier's legal standing-whether qualified to operate as supplier/vendor of the proposed goods services or works or not.*

The committee shall cause to be prepared the minutes of the meetings

The minutes shall include:

- A register of attendance
- Date of the meeting
- List of all matters considered and decision made, reasons for rejections, clarifications, minor amendments
- A notes regarding evaluation
- Conflicts of interest declared and any dissenting opinion among members
- Such other records as may be necessary.

Evaluation Committee

- Established for each procurement within threshold of TC
- May comprise separate technical & Financial evaluations or Combined
- At least 3 members recommended by Proc. & appointed by CEO
- Technical evaluation in strict adherence to compliance & evaluation criteria as set out in tender docs
- Performed with due diligence within 30 days
- Independent evaluation by each member before sharing the ratings
- Financial evaluation within 5 days of completion of technical evaluation

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Inspection & Acceptance Committee

- Composed of 3 members recommended by Proc. & appointed by CEO Shall:
  - inspect & test goods received
  - ensure compliance with terms & conditions of contract
  - accept or reject the deliveries
  - ensure correct quantity is received
  - goods meet technical stds
  - timely deliveries or note the delays
  - all manuals are received
  - issue interim or completion certs or GRNs

MEASURES FOR REDRESSING GRIEVANCES

Appeal Right

To redress meritorious grievances of candidates and correct system failures.

An Administrative Review Board may grand or recommend the following remedies:

- Declare the legal rule or principle that govern the subject matter
- Prohibit the procuring entity from acting or deciding unlawful
- Annul in whole or part an unlawful act or decision of a procuring entity.
- Revise an unlawful decision or substitute it with its own decision
- Administrative Review Board may grand or recommend the following remedies:
- Order that the procurement proceedings be terminated
- Review Board to make decision in stipulated timeframe.
- The decision to be final unless successfully reviewed by court.

Disposal of Stores and Equipment

What to be disposed of .

- **Stores and Equipment that are :**
  unserviceable,
  obsolete or
  surplus

Disposal methods

- As recommended by Disposal Committee subject to technical report

- **Disposal Committee**
  - To be established in accordance with the regulations

Disposal Methods

- transfer to another entity
- sale by public tender
- sale by public auction
- destruction, dumping or burying

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- trade-in

  There shall be a time limit for acceptance of recommendations of disposal committee by the Accounting Officer.

  Restriction on disposal to employees unless as prescribed in the regulations.

MANAGING STORES

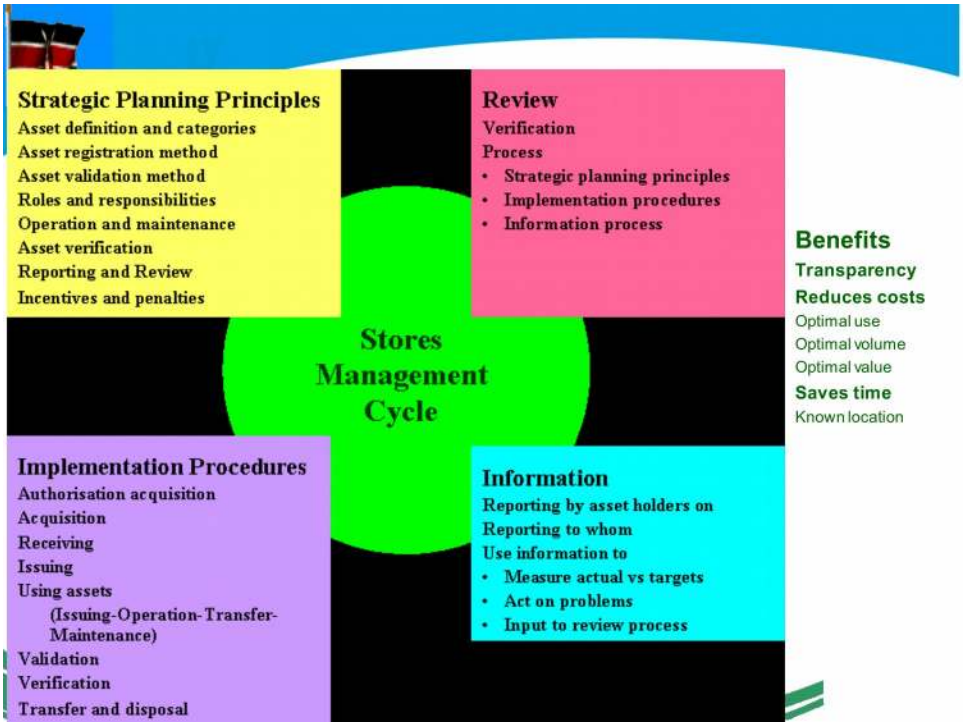
Items stored include:

a) Consumables – used within a short time(cotton wool, sutures, syringes etc)

b) Non-consumables – permanent and can be used for years

Procedure for equipment: deciding what to buy, ordering equipment, storing (recording, labeling and holding), issuing, controlling, maintaining and repairing

Store Management = Avail goods and equipment - in time -at the right place - at minimal costs



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Stores Management Cycle Strategic Planning ¼
Strategic decisions on principles:
Stock levels policy
Stock storage system (Centralisation level)
Registration system
What finance is needed/available?
Stock validation method (e.g. at cost price, depreciated, etc.)
Outsourcing or in-house
Institutional organisation

Stores Management Cycle Strategic Planning 3/4

Based on an **assessment** of:

- What goods and/or equipment are needed?
- Legal and regulatory requirements
- How does stores impact performance? How measured?
- Organisation set-up
- Human resource requirements
- Risks and costs
- In-house or outsourced?

Stores Management Cycle Strategic Planning 4/4
Stores design
Decide location(s)

- Based on type stock (cold, size, climate, hazardous, sensitive)
- When needed and where?
- Legal and regulatory requirements
- Access

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- Security and safety (hazardous goods, fire, accident, health)

Decide Storage/stockyard lay-out & equipment

- Legal and regulatory requirements
- Conditions for storing goods and equipment
- Access and efficiency of movement
- Energy supply - emergency back-up needed?
- Security measures to be installed

Human Resource Management

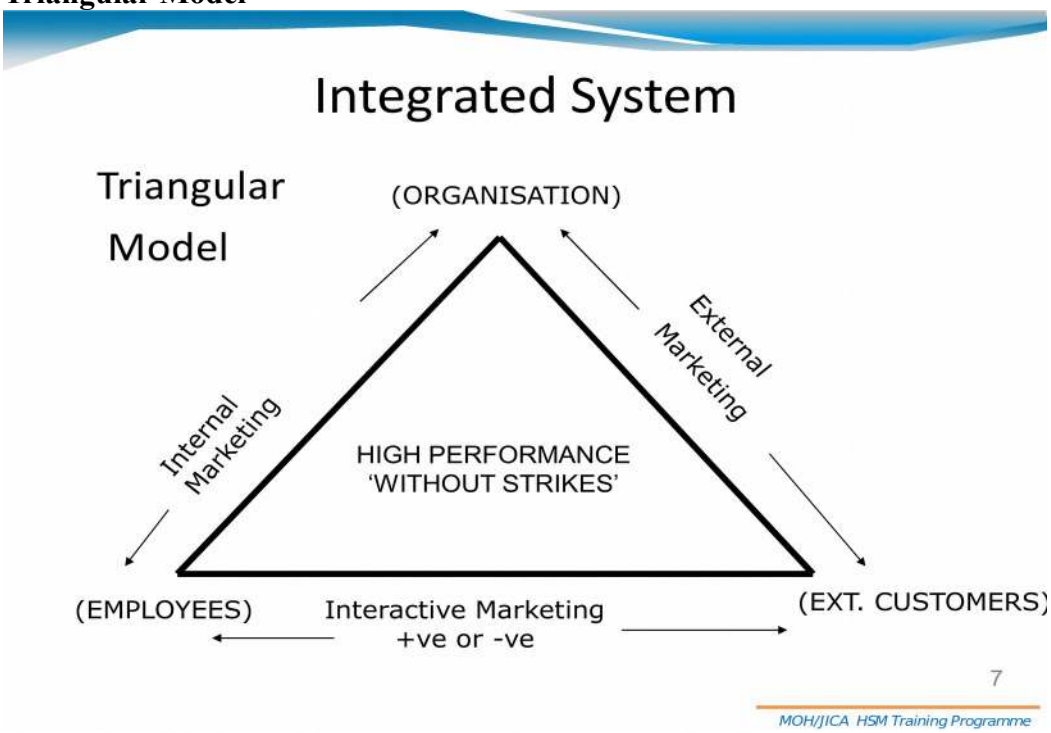
- Detailed organogram and job descriptions
- Recruit and manage staff (Number, qualifications, motivation, training, supervision, monitoring)

PUBLIC RELATIONS AND CUSTOMER CARE

Customer Relationship Management (CRM) is a social process of managing all networks of communication to ensure mutual understanding between the service provider (health worker) and the customer (e.g. patient from OPD or Nurse in the Theatre).

Relationships are preceded by social communication for both internal and External Customers. Communication in this context relates to both verbal and nonverbal. In order to achieve effective communication there is a need to ensure the existence of the 'integrated marketing system' (Triangular model).

Triangular Model



Integrated System Interactive Marketing +ve or -ve (EMPLOYEES) (EXT. CUSTOMERS) (ORGANISATION) HIGH PERFORMANCE

The Customers in Health System

The customers in health systems can be categorized as internal customer and external customers.

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Internal Customers
Internal customers include all individuals, departments, sections within the organization who use the output of your job (e.g. Pharmacist & the Accountant of the Hospital). The satisfaction of the internal customers is crucial to the achievement of the organizational goals and positive image. Internal customer relationship is a crucial part of CRM as when it fails it gives room for Gossips that distorts the image of the organization.
External Customers
External customers include individuals, organizations and anybody outside the organization who buy/use/ the goods and services provided by the organization. In the case of Health service; suppliers, patients, regulatory bodies and the like form the external customers.
The concept of Integrated Marketing targets the employees, employers & external Customers given that it highlights the roles of each of the above mentioned. It is important to note that a customer may belong to various categories.
Categories of Customers
• Internal Vs External
• Industrial Vs Individual
• Existing Vs New
• Physical Product Customers Vs Service Customers
• Short term Vs Long term
• General Vs Prestigious (VIP)

Types of customers in the individual markets (human-human interaction)
The silent (uncommunicative) Customer
Is shy, quiet or deliberately uncommunicative; scarcely talks (e.g. a patient with an STD)

The talkative Customer
Is friendly and has a sense of humor, willing to talk about different issues

The sarcastic (over-familiar) Customer
Talks a lot; often fast, without stop, could be the most difficult customer (e.g. a doctor who is a patient)

The good-nurtured Customer
Says yes to almost everything but will not make a decision (e.g. a Nurse who fears a doctor or a doctor with his boss
The angry-bad tempered Customer
Is irritated with something small (trivial)
The hostile, rude, argumentative Customer
Very difficult customer; she/he always attacks you on whatever you say to challenge your position (a Pharmacist when procuring the drugs)
The aloof/disinterested customer
Similar to the silent customers, very reserved.
The busy Customer (Impulse Customer)
Appears busy, has hurried movements and air of anxiety and can't concentrate on service offered at the counter.
The invisible Customer/Telephone Customer

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This is a customer you can`t see and must be handled carefully, has never complained.

The dictatorial Customer

Has a power complex and wishes to put you at a disadvantage. His/her actions are discourteous and intolerable.

The hesitant/suspicious/uncertain Customer

Is challengeable, uncertain and lacks confidence.

The 'woman' type (or men type!) customer.

Focus on high quality with low price

Expectations of Customers

Every time clients come for health services they have expectations. Consumer expectations are an important factor when considering improvement of health systems management. The product of Health Systems Management is consumer satisfaction, which can only be achieved if the following are observed in the interest of the client:

- Speed and timeliness of service delivery/Punctuality
- Courtesy and helpfulness
- Service reliability
- Quality Service
- Consistency in service delivery
- Accuracy of paperwork
- Positive attitude from staff
- User-friendly systems/
- less bureaucracy
- Aesthetic environment

In order **to improve customer relationship management** at any level, it is necessary to know your customers, their needs and expectations very well (Carlzon, 1989). At the same time it is important to be clear on your role in order to meet your consumer demand to the level of their expectation.

The Value of the Relationship

Customer relationship management is about adding value to the relationship between the service provider and the client/customer. The result of value added relationship include increased loyalty of the customers, increased customers engagement in marketing the organization and finally improved performance and synergy on the basis of team spirit.

Internal Relationship Marketing System

Internal relationship management system recognizes the application of networking in working relationships between the organization and its customers. All employees are customers of managers who wish to carry out the departments` objectives.

The crucial aspects of internal relationship marketing system include the employer's engagement with those who create value as well as the need to brand the organization positively both inside and outside. Integrating psychological contract built from social capital reduces insecurity by filling in the gaps in the relationship not covered by a formal written employment contract. It is important to display commitment on the part of the organization to care for the personal and

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social needs of the employees who are customers of the managers who wish to carry out the firm's objectives.

DISASTER MANAGEMENT

At the end of this unit, the learner should be able to:

1. Define the various terms related to disaster management
2. Differentiate between disaster and hazard
3. Explain the elements of risk in relation to disaster management
4. Describe the types of disasters
5. Explain disaster management stages
6. Explain disaster management process
7. Describe the five (R) risk strategies involved in disaster management
8. Explain the effects of disasters on health
9. Describe vulnerability in relation to disaster management

Common terminology used in disaster management

A Hazard: A hazard is a dangerous phenomenon, substance, human activity or condition that may cause the loss of life, injury or other health impacts, property damage, loss of livelihoods and services, social and economic disruption, or environmental damage.

A Natural Hazard: Natural processes or phenomenon that may cause loss of life, injury or other health impacts, property damage, loss of livelihoods and services, social and economic disruption, or environmental damage.

A Geological Hazard: Geological process or phenomenon that may cause loss of life, injury or other health impacts, property damage, loss of livelihoods and services, social and economic disruption, or environmental damage.

Technological Hazard: A hazard originating from technological or industrial conditions, including accidents, dangerous procedures, infrastructure failures or specific human activities, that may cause loss of life, injury, illness or other health impacts, property damage, loss of livelihoods and services, social and economic disruption, or environmental damage.

A Disaster: A disaster is a serious disruption of the functioning of a community or society causing widespread human, material, economic or environmental loses which exceed the ability of the affected community/society to cope using its own resources.

A Disaster Risk: The potential disaster losses, in lives, health status, livelihoods, assets and services, which could occur to a particular community or a society over some specified future time period.

Disaster Risk Management: The systematic process of using administrative directives, organizations, and operational skills and capacities to implement strategies, policies and improved coping capacities in order to lessen the adverse impacts of hazards and the possibility of disaster.

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Disaster Risk Reduction: The concept and practice of reducing disaster risks through systematic efforts to analyze and manage the causal factors of disasters, including through reduced exposure to hazards, lessened vulnerability of people and property, wise management of land and the environment, and improved preparedness for adverse events.

Risk Assessment: A methodology to determine the nature and extent of risk by analyzing potential hazards and evaluating existing conditions of vulnerability that together could potentially harm exposed people, property, services, livelihoods and the environment on which they depend.

Climate Change: A change in the state of the climate that can be identified (e.g. by using statistical tests) by changes in the mean and/or the variabilities of its properties and that persist for an extended period, typically decades or longer. Climate Change may be due to natural internal processes or external forcing or to persistent anthropogenic changes in the composition of the atmosphere or land use.

Adaptation to Climate Change: The adjustment in natural or human activities in responses to actual or expected climate stimuli or their effects which moderate harm or exploit beneficial opportunities.

Vulnerability: The characteristics and circumstances of a community, system or asset that makes it susceptible to the damaging effects of a hazard.

Disaster occurs when a hazard results in devastation that leaves communities or even whole nations unable to cope unaided but disasters are neither inevitable nor 'natural'. There is no such thing as a 'natural' disaster. Many hazards are natural and usually inevitable, like cyclones, floods, droughts and earthquakes. They are 'hazards' in that they can potentially harm people, economies and the environment if they are not adequately prepared.
Moreover, the world has witnessed an alarming increase in the frequency and severity of disasters. Disaster cases are rising throughout the world due to a number of factors that include:

- more frequent extreme weather events
- agricultural production systems that increase risk
- population growth combined with demographic change and movements leading, for instance, unplanned urbanization, growing demand for food, industrial goods and services; and
- Over-exploitation of natural resources.

While better emergency response systems will save lives and properties, many of these losses can be avoided – or reduced – if appropriate policies and programmes are instituted to address the root causes and set in place **mitigation, preparedness and response mechanisms** that are effectively integrated into overall development planning.
These issues were debated during the World Conference on Disaster Reduction (WCDR) in Kobe, Hyogo, Japan (January 2005). It was therefore insisted on the need to move from theory to concrete action in disaster risk reduction.

Hazard and Disaster:

– closely related terms

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    - However, a hazard is a natural event while the disaster is its consequence
    - Hazard" refers to the event which may cause disaster
    - They could be either man-made or naturally occurring hazards in our environment

Disaster is combination of two words: **'des'** meaning evil and **'astre'** meaning star

- Disaster therefore means **evil star.** A complete definition of disaster provided by Turner (1976) as 'an event, concentrated in time and space, which threatens a society or a sub-division of a society with major unwanted consequences as a result of the collapse of precautions which had hitherto been culturally accepted as adequate'
- Common definition: major misfortune which disrupts the normal functioning of the society
- **Emergency:**
    - The term 'emergency' is used to describe the crisis which a community has great difficulty to cope with
    - External assistance is needed to cope with the emergencies
    - political emergency, drought, famine etc.
- **Calamity:**
  Calamity is defined as deep distress, disaster to a lesser degree of damage, which disrupts the normal functioning of the people of a society
- **Vulnerability:**
    - "Vulnerability" basically means susceptibility of a population or system (e.g., a hospital, water supply and sewage system, or aspects of infrastructure) to the effects of the hazard
    - "The extent to which a community, structure, service or geographic area is likely to be damaged or disrupted by the impact of particular disaster hazard…"
    - Its magnitude is expressed on a scale from 0.0 (No damage or loss) to 1.0 (total loss).
- **Risk:**
    - The probability that a particular system or population will be affected by hazards is known as the "risk."
    - Risk is a function of the vulnerability and the hazard, and is expressed as follows:
        - Risk = Vulnerability x Hazard. It means, the risk increases if the vulnerability of the population or the systems is more; and also of the magnitude of the hazard is more.

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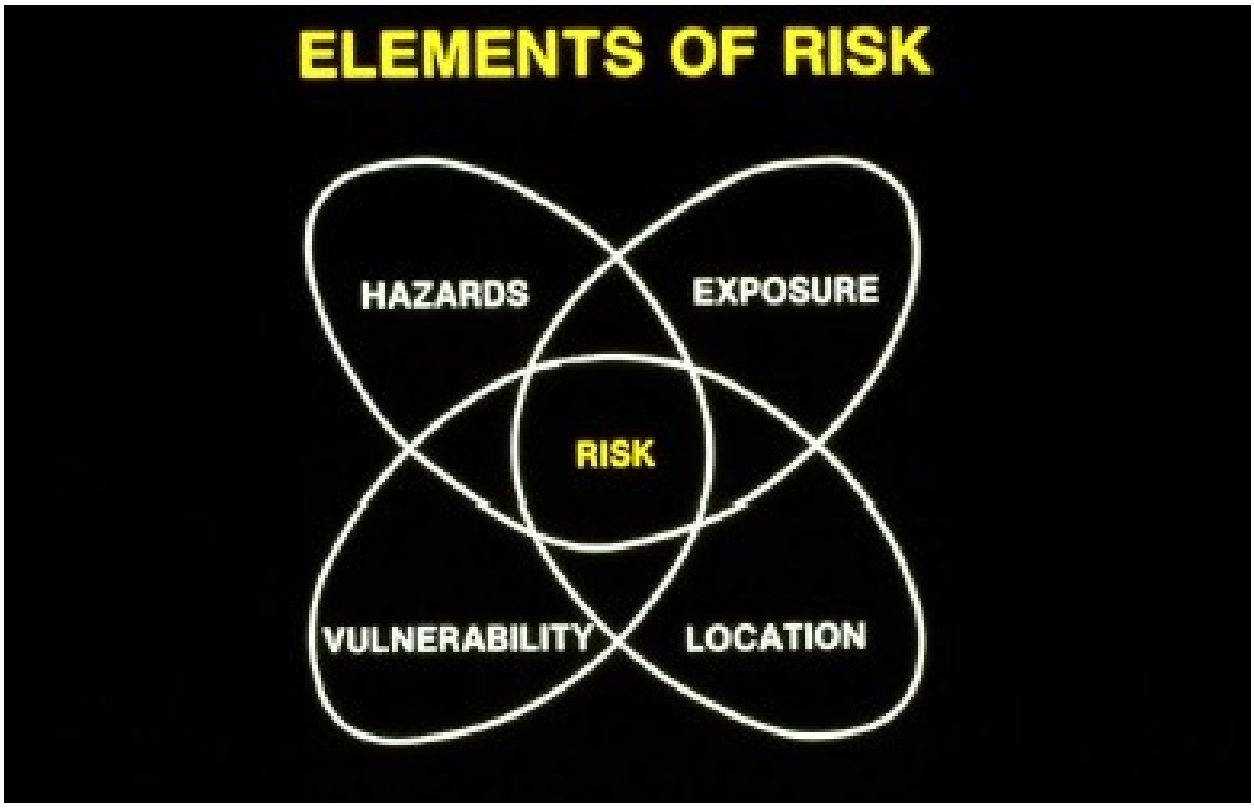

Characterization of Disasters

Types disasters:

Various types of disasters have been categorized under five sub groups

- ***Sub-Group 1****:* Water and Climate Related Hazards.

  (i) Floods, (ii) Droughts, (iii) Cyclones, (iv) Tornadoes & Hurricanes, (v) Hailstorm, (vi) Cloudburst, (vii) Snow Avalanches, (viii) Heat & Cold Waves, (ix) Sea Erosion, (x) Thunder and Lightening
- ***Sub-Group* 2**: Geologically related Hazards

  Earthquakes, (ii) Landslides, (iii) Mudflows, (iv) Dam Bursts, (v) Dam Failures
- ***Sub-Group* 3**: Chemical, Industrial & Nuclear related Disasters
- ***Sub-Group* 4**: Accident Related Disasters.

  Road, Rail and other Transportation accidents, (ii) Major Building Collapse, (iii) Serial Bomb Blasts, (iv) Festival related Disasters, (v) Fires, (vi) Forest Fires
- ***Sub-Group* 5**: Biologically Related Disasters

  (i) Biological Disasters, (ii) Epidemics, (iii) Cattle Epidemics, (iv) Pest Attacks, (v) Food Poisoning

Similarly, disasters can be classified as follows:

1. **Anthropogenic hazards**: These are human made hazards such as
   a. anthropogenic pollutant hazards which come as a result of transformation of the nature by human beings e.g. production of chemical,
   b. anthropogenic pollutant hazards which come as a result of transformation of nature by human being e.g. nuclear plant, industrial pollution

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2. **Environmental degradation**. These hazards can be anthropogenic, socio-natural or natural
3. **Biological hazards.** These hazards include anthropogenic, socio-natural or natural. Examples are: epidemics, pandemics, locusts bioterrorism/biological weapons
4. **Geological/geotectonic origin:** The category include land movement, volcanoes, earthquake and tsunami
5. **Geological/geomorphism origin**: These include land slide, avalanche/snow slide, land and coastal erosion
6. **Meteorological and climate origin**: The include hurricanes, tropical storms and tornados
7. **Hydraulic origin**: food, water stagnation

The last two can be combined together as hydro-meteorological

8. **Socio-natural hazards**. Examples are: Flood, Desertification, Drought, Deforestation, Climate change and Global warming
9. **Occupational/ergonomic hazards**. These are occupational accidents arising due to work position at work place.
10. **Physical hazards:** these are heat, cold, global warming and climate change
11. **Mechanical hazards**. These are accidents
12. **Psychological hazards**. Examples are stress, schizophrenia, anxiety, depression and mental disorders
13. **Agricultural hazards**. Pesticides, fertilizers

What is understanding hazard?

Understanding hazard involves:

- How hazards arise
- Probability of occurrence and magnitude
- Physical mechanism of destruction
    - Eg. In earthquakes, most fatalities are due to building collapse. Focus should be on prevention of building collapse
    - In floods, death occurs due to drowning in fast flowing currents. Focus is on limiting the exposure of people by keeping them out of track of water
- Elements most vulnerable
- Consequence of damage
- Damage to factory can lead to jobless people

<u>AIMS OF DISASTER MANAGEMENT</u>

- Reduce (Avoid, if possible) the potential losses from hazards.
- Assure prompt and appropriate assistance to victims when necessary.
- Achieve rapid and durable recovery**.**

<u>Stages of disaster management</u>

- Pre-disaster phase: taken prior to the occurrence of any disasters. For. E.g. Construction of buildings which can withstand the impact of disaster. It is also called 'mitigation phase'.

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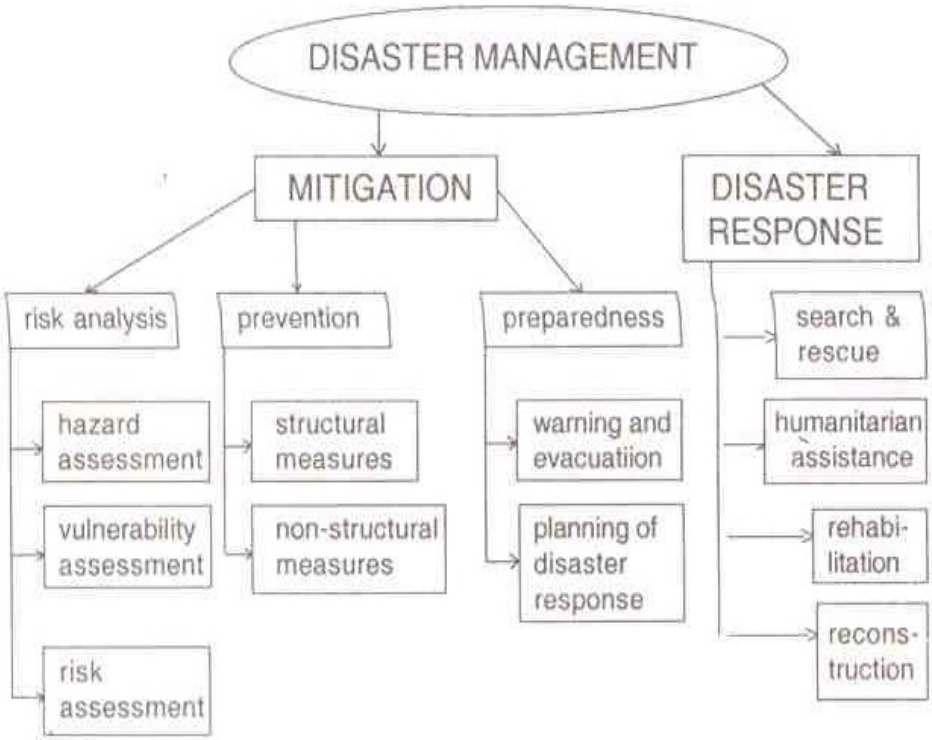
- Actual disaster phase: During this phase, timely warning is to be given to the people so that they can timely take action to face the disaster. It is also called the 'preparatory phase'.
- Post-disaster phase: It includes the measures that are taken after the occurrence of a disaster. It further includes three phases:
    - Relief phase: immediately after the occurrence of a disaster.
    - Rehabilitation phase: to rehabilitate the community through the development of house, restoring water supply, sanitation, food and providing loans so that the people are brought back to the normal work.
    - Reconstruction Phase: It involves developing new colonies to make the disaster affected people to settle permanently.



"Five-R Strategy"

Disaster management is based on "Five-R Strategy" of

- Rescue
- Relief
- Restoration
- Rehabilitation
- Reconstruction

Disaster risk management

Preparatory phase

- The preparatory phase of disaster response includes all of the activities that help a society and the disaster agencies to prepare for a disaster event. These activities are broadly classified as:

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- disaster prevention:
  - The objective of prevention is to prevent the disaster from occurring.
- disaster mitigation
  - Disaster mitigation accepts that some natural event may occur, but it tries to lessen the impact by improving the common ability
- disaster preparedness.
  - Disaster preparedness assumes that a disaster will occur; it focuses on structuring the emergency response and on laying a framework for recovery.

Disaster mitigation

- Disaster Mitigation is essentially measures taken in advance of a disaster aimed at minimizing or eliminating the impact of disaster on community and environment
- 2 components: reducing hazard and reducing vulnerability

Reducing hazard vs reducing vulnerability

- **Reducing hazard:**
  - protection against threat by removing the cause of threat
  - Eg. Construction of levees along banks of river to reduce chance of flooding
  - Tree plantation
- **Reducing vulnerability:**
  - reducing the effect of threat
  - Eg. Constructing buildings as per bylaws in NBC
- **Most of the natural hazards are unpreventable, therefore, we mostly work towards reducing vulnerability**

Disaster Mitigation Measures:

- Passive mitigation measure
- Active mitigation measures

Passive mitigation measures:

Authorities prevent undesired actions through controls and penalties by:

- Requirement to conform with design codes
- Checking compliance of controls on-site
- Imposing court proceedings, fines, closure orders on offenders
- Control of land use
- Denial of utilities and infrastructure to areas where development is undesired
- Compulsory insurance

Requirements of passive control systems

a. An existing and enforceable system of control
b. Acceptance by the affected community of the objectives and the authority imposing the controls
c. The economic capability of the affected community to comply with the regulations.

Active mitigation measures

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Authorities promote desired actions through incentives like:

- Planning control dispensations
- Training and education
- Economic assistance (grants and preferential loans)
- Subsidies on safety equipment, safer building materials, etc.
- Provision of facilities: safer buildings, refuge points, storage
- Public information dissemination and awareness raising
- Promotion of voluntary insurance
- Creation of community organizations

Active Programs

a. Aim to create a self-perpetuating safety culture in areas of weak authority or poor ability to comply with existing controls.
b. Require large budgets, skilled manpower and extensive administration.
c. Are useful in areas of low income, rural areas or elsewhere where there is no external jurisdiction over land use or building activity.

Phases of disaster mitigation

- Risk analysis
- Prevention
- Preparedness

Risk Analysis:

- What is risk?
  - Expected loss caused by a particular phenomenon
- Risk analysis:
  - Process of determining the nature and scale of losses which can be anticipated in a particular area**.**
  - **It involves analysis of**
    - the probability of a hazard of a particular magnitude occurring (called hazard assessment)
    - the elements susceptible to potential loss or damage (called elements at risk)
    - the nature of vulnerability of those elements (vulnerability analysis), and a specified future time period (loss estimation)

Hazard assessment

- The process of estimating the probability of occurrence of a damaging phenomenon of given magnitude in a given area
- Based on collection of historical and scientific data
- Points to be considered in Hazard assessment:
  - History
  - Probability of various intensities
  - Maximum threat

Possible secondary hazards: eg. Earthquake can cause dam burst

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Elements at risk:

- People: loss of life,
- *Personal* health: injury or disease,
- Damage, destruction of property,
- Damage, destruction of infrastructure, public service systems,
- Damage, destruction of environment,
- Damage, destruction of crops,
- Disruption, loss of production,
- Disruption, loss of livelihood,
- Disruption, loss of essential services,
- Disruption, loss of *national* infrastructure,
- Disruption to governmental process, systems,
- Loss to national, local economies,
- Disruption, loss of community or lifestyle, and
- Sociological and psychological consequences.

Effects of disaster on health:

1. Population density
2. Population displacement
3. Disruption and contamination of water supply and sanitation services
4. Disruption of public health programme
5. Economic changes that favour breeding of factors
6. Displacement of domestic and wild animals
7. Provision of emergency food, water and shelter in disaster situation
8. Climate change and health issues

Disaster building block:

1. Disaster/impact
2. Response
3. Rehabilitation
4. Reconstruction
5. Mitigation
9. Preparedness

In comprehensive emergency management model (CEMM), there should be disaster preparedness in such as way that if there is a disaster, there is effective disaster response and therefore, able to deal effectively with rehabilitation and reconstruction. Moreover, disaster mitigation is crucial in dealing with disaster.

Vulnerability analysis:

- A process which results in an understanding of the types and levels of exposure of persons, property, and the environment to the effects of identified hazards at a particular time.
- Vulnerability = People + Condition + Place + Time + Event
- vulnerability also differs according to intensity of the hazard

Types of Vulnerability:

- Physical Vulnerability
- Economic

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- Social
- environmental

Physical Vulnerability:

- pertaining to matters of location, structural and infrastructural conditions etc
- **Includes**
  - Buildings at risk
  - Unsafe infrastructure: eg. Transportation system, sanitation system etc.
  - Unsafe critical facilities
  - Rapid urbanization
  - Agricultural-primarily considering physical assets-related, but opportunity loss potential of essential natural resources, crops, trees, livestock, fisheries, also should be recognized.

Economic Vulnerability:

- Determined by evaluating
  - the direct loss potential of economic assets
  - indirect loss potential
  - secondary effects
- ***Direct Loss Potential***
  - destruction of buildings, plants, facilities, raw material, products,
  - Replacement costs,
  - Loss of employment,
  - Crop *losses,* and
  - Damage to means of production.
- ***Indirect Loss Potential***
  - Impact of lost production,
  - Impact of lost employment,
  - Loss of markets,
  - Loss of opportunity,
  - Loss to consequential income-earning activities.
- ***Secondary Effects***
  - Inflation,
  - Indebtedness,
  - Labour migration**,**

Social Vulnerability:

- determined by the perception of risk and the ability of people to take measures to reduce that risk
- more difficult to measure than either physical or economic vulnerability
- **Critical Indicators of Perceived Risk, Ability of Response:**
  - Poverty, limitation of resources, reserves or options,
  - Degree of public awareness about the immediate social and physical environments
  - Prior personal experience of specific risk, consequence.
- **Demographic Considerations:**
  - Magnitudes, total population,
  - Concentration densities,

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- Demographic distinctions, vulnerable groups of socially disadvantaged people or those requiring special attention. These include children, disabled, elderly people, women

Capabilities:

When conducting vulnerability analysis, special note should be taken of positive attributes able to be identified that may contribute to an enhanced ability to prevent or mitigate the effects of a disaster, or which may strengthen a community's ability to respond effectively to the hazard

Technocratic view for managing the risk

- **Problem-**Physical vulnerability
- **Symptoms as perceived-** High death toll and damages of infrastructure attribute to severity of hazard, extensive and recurrent rehabilitation needs.
- **The causes-** uncontrolled characteristics of hazard events, physiography of the prone areas, lack of inadequacy of protective infrastructure, failure of forecasting and warning.

Risk Management Process

Risk management has three components namely:

- Risk identification
- Risk reduction
- Risk transfer

Risk identification has to be done through mapping and using other available technological options

It is usual to allocate risk management to special body at national level. Usually, it is a national disaster management organization at national level. Others support should be obtained within a National Action Plan for disaster management.

At local, it may be the responsibility of a disaster mitigation committee, which administers risk management.

Risk reduction:

Effective risk reduction involves mitigation measures in hazard prone areas. It may also involve overcoming the socio-economic, institutional and political barriers to the adoption of effective risk reduction strategies and measures in developing countries.

Risk Management:

This is the process by which assessed risks are mitigated, minimized or controlled through engineering, management of land use prances or other operational means. This involves the optimal allocation of available resources in support of group goals

Risk Transfer:

Effective risk transfer involves different tools such as insurances; tax polices special measures focused on land management. Organizational structures, policies, legislation etc is required for effective implementation of risk transfer strategies for a country or local government areas.

Steps in Hazard analysis

- Understand the type and nature of hazard
- Collect the data from history about frequency, duration, controllability etc.
- Vulnerability analysis: physical, economic etc.
- Strategies to minimize vulnerability

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Disaster Mitigation Actions:

- Engineering and construction
  - Engineered buildings
  - Non-engineered
  - Existing buildings
- Physical planning
  - Location of public sector units
  - Provision of several small facilities
  - Changing use of vulnerable buildings
  - Availability of safer land for poorer community
- Economic
  - Diversification of economy: single type of industry is vulnerable
  - Economic incentives like loans, tax concessions etc.
- Management and institutional
  - Special department for disaster management
- Societal
  - Public awareness through education
  - Involvement of community through drills

Assignment/sample questions

1. Map out disasters in Kenya for the last 10 years
2. Describe the trends of various disasters in Kenya for the last 10 years
3. Identify the current Kenyan legislations on disaster management
4. Explain the application of compressive emergency management model
5. Describe the following in relation to disaster management:
   a. Disaster
   b. Response
   c. Rehabilitation
   d. Reconstruction
   e. Mitigation and
   f. Preparedness

PROJECT MANAGEMENT

Planning

Planning is making decisions about which courses of action to follow. It is the root to objective where it involves conceptualizing and anticipating future action

It includes the following activities:

- Establishing objectives to determine the end result;
- Developing strategies to determine how to reach objectives, by when, and by whom;
- Preparing budgets to determine the cost of using resources;
- Establishing policies to have standing decisions on recurring situations;
- Establishing standards to ensure continued quality of services and products.

Importance of planning

- Gives a sense of direction.

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- Focuses attention on objectives.
- Involve others –participation, teamwork.
- Helps anticipate problems and cope with any change in an organization.
- Enhances appropriate allocation of resources.
- Helps in coordination
- Leads to more effectiveness and efficiency of services

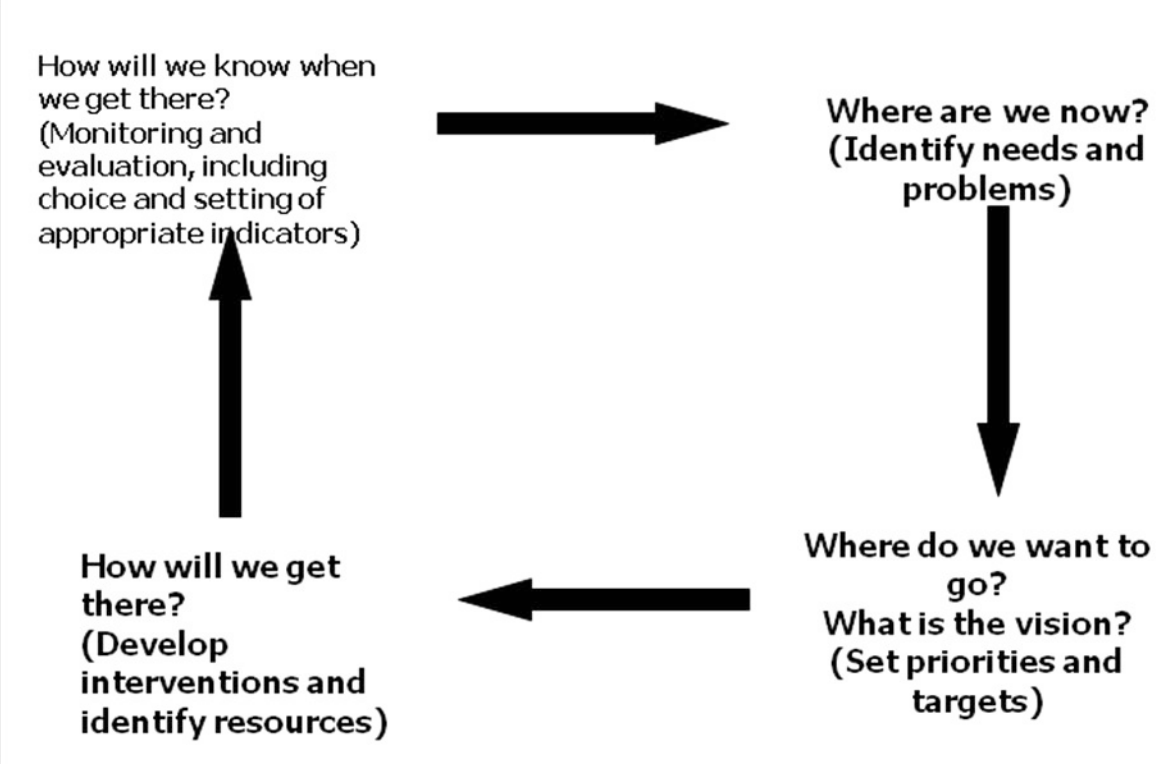
Key elements of planning

- Knowing where we are (Situation)
- Deciding where we want to go (Vision/Objectives)
- What do we need to reach where we want to go (Resources)
- How we are going there (Method/process/how)
- Deciding how far we can get towards your target in a period of time (Time line/When)
- How will we know when we get there?

This requires the development of measurable indicators for monitoring progress and evaluating results

- How far we have gone
- Where we are not on target
- How we can do better in future
- Amending the implementation plan (RE-planning

THE PLANNING CYCLE



Who should plan?

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Planning should be a participatory process

- Community /key stakeholders must be involved through the process, e .g in doing the contextual and situational analysis, setting priorities e.t.c.
- Basic components of every organization with which the manager must work are people, things and ideas.

Rationale for Participatory planning

- Mobilization and allocation of resources to different health problems
- Participation is open-ended hence allows greater participation (*Ownership and sustainability)*
- Guide the anticipation of potential constraints and solutions to achieving the vision – systematic and predictable way
- Promote synergy and greater coordination of health problems in a multi-stakeholders' environment

Key Steps in Planning

1. Situation analysis
2. Setting priority
3. Options appraisal
4. Programming
5. Implementation and Monitoring
6. Evaluation

Situation analysis

Improving the understanding of the current situation in order to:

- Provide a common reference point for the planning process
- Allow the selection of priority areas of concern for planning

SA outlines and assesses 3 broad areas

- Demographic situation and health needs
- Services and resources
  - Socioeconomic Status
  - Health related services of other sectors
  - Geographic and topographic pattern
  - Present infrastructure and service pattern
  - Existing service resources
  - Effectiveness and efficiency of current services
- Policy and political environment (General, Govt. Policies)

Setting priority

- Making choices among major needs so as to allocate resources efficiently.
- Consider the following in setting priority
  - Magnitude of the issue/Problem
  - Who is determining the needs
  - Target group
  - Cost, ease and effectiveness of intervention

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- Political context

Options appraisal

- Process of deciding between alternative approaches to achieve an objective
  - Determine the objectives of the activity being appraised
  - Identify alternative options (exclude unfeasible and uneconomical option)
  - Identify resources required to achieve each option and the benefit of each option
  - Cost the resources required

- A good option appraisal should;
  - Present quantifiable and non-quantifiable costs and benefits
  - Explicitly recognize and incorporate the need for other perspective
  - Recognize the resource and financial implication
  - Examine the technical feasibility
  - Consider the process effects
  - Set out information on the methodology used
  - Perform where necessary sensitivity analysis
- Set out key assumptions
- Use of sustainability Grid

Programming

- Budgeting and Developing of a plan of action
- This process is carried out for each priority area and its associated set of targets.
- It translates the results of the earlier decisions into a series of programs each with a budget and resources

Common Planning Tools

- Logical Framework
- Gantt Charts
- Activity Schedule

Logical Framework

A tool to help designers of projects think logically about:

- what the project is trying to achieve *(Purpose),*
- what things the project need to do to bring that about *(Outputs)*
- what needs to be done to produce these outputs *(Activities)*
- What is required to implement the activities (Input)
- What potential problems could affect the success of the project
- How the progress and ultimate success of the project will be measured

Objectives	Objectively Verifiable Indicators	Means of Verification	Assumptions and Risks
Overall Goal			
Project Purpose			
Results / Outputs			

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Activities			

Advantages of Log Frame

- It brings together in one place a statement of all the key components of a project
- It presents them in a systematic, concise and coherent way, thus clarifying and exposing the logic of how the project is expected to work
- It separates out the various levels in the hierarchy of objectives, helping to ensure that inputs and outputs are not confused with each other or with objectives and that wider ranging objectives are not overlooked
- It clarifies the relationships which underlie judgments about likely efficiency and effectiveness of projects
- It identifies the main factors related to the success of the project
- It provides a basis for monitoring and evaluation by identifying indicators of success, and means of quantification or assessment

Gantt Charts

- A special chart used by project planners to illustrate the project schedule from the start to the end
- Shows the broad work breakdown structure of a project

Project Activity Schedule

- Simple Matrix outlining the specific activities, when they will be done, who is responsible and resources required.

What(Activity)	Where (Site)	When (Time Frame)	Who (Responsible person)

Budgeting

- Determination of cost of inputs and activities required to implement a plan
  - Personnel costs
  - Non labor charges: equipment, travel, reproduction of materials, computer use, office space, supplies etc
  - Administrative costs
  - Overheads eg. Staff benefits etc

Implementation and monitoring

Implementation is the process of transforming the broad program into more specific timed and budgeted sets of tasks and activities. It is the action portion of the plan. Drawing up a more operational plan or a work plan and taking action accordingly.Implementation involves securing community participation for launching the project, co-ordination of activities, monitoring, and

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taking care of contingencies. Project implementation includes various management functions Including

- Co-ordination
- Organizing
- Staffing
- Controlling
- Team-building
- Monitoring
- Contingency Management

- Monitoring – Continuous assessment of the progress of an activities over time. Observes whether an activity or service is occurring as planned based on activity plans and schedules

A good monitoring system ensures that the programme interventions stay on course by:

- checking that activities are implemented
- measuring progress towards objectives
- identifying problems as they come up
- identifying strengths that can be built on.
- adapting to changing circumstances.
- It uses the systematic collection of data on specified indicators to inform management and the main stakeholders of an ongoing operation of the extent of progress and achievement of results in the use of allocated funds.

EVALUATION

Definition: Evaluation means measurement, appraisal, or making judgments on the output and impact of the project in terms of the objectives.

It is the periodic assessment of the relevance, performance, efficiency, effectiveness, impact and sustainability, and benefits to the target community

Evaluations are conducted to determine whether a project has met its objective/goals.

The process of determining whether the program is achieving it's purpose, whether it should be continued or terminated, and how it can be improved or better managed.

The purpose of evaluation

- To improve health programmes and the services of delivering them
- Guide the allocation of financial and human resources
- Provides a basis for the next assessment hence a fresh lead in to the planning spiral
- Evaluation is used to determine whether
  - The activities were worth carrying out
  - The activity should be continued or not
  - It accomplished the objectives it set out to achieve
  - It should be extended here or elsewhere
  - Modification of the activity might make a difference to its performance
  - To justify spending on an activity

EVALUATION CYCLE

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1. Define the purpose of the evaluation
Who wants the evaluation, why and what how are they going to use the results
2. Define the priority areas to be evaluated. This includes the most important area to be evaluated – e.g projects' progress towards reaching its goals and objectives, technical aspects and so on
3. Information collection plan: Use the focus areas and list:
What key factors (indicators) will be researched on each area,
The source of information (i.e. the data collection tool and where that information is to be found)
4. The implementation plan.

- Decide who will participate.
- Make a budget.
- A list of participants,
- Detailed calendar of tasks and responsibilities, including a target date for report completion,
- If instruments (interviews, surveys, etc.) are needed, these can be designed immediately after the planning session, and field-tested according to the plan.

5. **Analyse and organize the information**.
How is the information to be analysed and by whom? Who will develop the final conclusions and recommendations? Make an outline of the final report. Decide how much data will be needed, and what limits of time and other resources affect the work of tabulation.

6. Communicating the findings and making follow- up
A follow-up plan should include

a). An agreement on specific tasks/actions to be taken by the donor agency, the project implementer, and the beneficiary organization to meet the evaluation's recommendations,
b). Designation of persons to do each follow-up activity,
c). An estimate of dates for implementing each activity
d). The date when the activity is to be completed.

Types of evaluation
1. Process evaluation
Internal evaluation – periodic self evaluation of a project including review of goals, strategies and work plans. Answers the questions about : **HOW** the program is implemented and how the outcomes are achieved

Components of process evaluation
Relevance – rationale for adopting the community based health care policies and practices.
Measures the response of the project to essential human needs and social and health policies and priorities
Adequacy – sufficient attention has been paid to certain previously determined courses of action
Process – comparison of actual with scheduled activities, the identification of reasons for achievements or short comings and indications for remedies for any shortcomings
Purpose is to facilitate the monitoring and operational control of on going activities

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Efficiency- an expression of the relationship between results obtained from a health program or activity and the efforts expended in terms of human, financial and other resources. – aims at improving implementation
Effectiveness – expression of the desired effect of a program, service institution or support activity in reducing a health problem. Measures the degree of attainment

2. Impact evaluation
Aims at identifying any necessary change in the direction of health programs so as to increase their contribution to overall health and socioeconomic development.

- Impact evaluation measures whether or not a project achieved its goals and attempts to see what impact the project had on participants
- An independent consultant may conduct the impact evaluation
- Scope includes all stake holders – donor agency, international cooperating agency, local project organization and the beneficially community

an expression of the overall effect of a program, service or institution on health and related socio-demographic development

INDICATORS FOR EVALUATION
Indicators are variables that help to measure changes.
The indicators for evaluation include:

- Health policy indicators which include
  - Political commitment
  - Resource allocation
  - Degree of equitable distribution of the resources
  - Level of community involvement

  Social and economic indicators

BENEFITS OF PROGRAM EVALUATION

- Reflect on progress; see where we are going and where we are coming from and improve programs
- Influence policy makes and funders
- Build community capacity and engage communities
- Benefits of program evaluation
- Share what works and what doesn't work with other communities
- Ensure funding and sustainability
- Strengthen accountability

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